

Eating Disorders Behavioral Health Levels of Care Comparison Chart, Child or Adolescent

MCG Health
Behavioral Health Care
28th Edition

This MCG guideline has not yet been updated to reflect new guidance and requirements found in the 2024 Medicare Advantage and Part D Final Rule (CMS-4201-F). For Medicare Advantage beneficiaries, before referring to MCG's guidelines users should first follow all guidance related to this topic located in CMS National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), and other statutes and regulations. Content used in MCG guidelines may be used to supplement but does not replace, modify, or supersede existing Medicare regulations or applicable NCDs or LCDs.

- Eating Disorders Behavioral Health Levels of Care Comparison Chart, Child or Adolescent
- References
- Footnotes
- Definitions

Eating Disorders Behavioral Health Levels of Care Comparison Chart, Child or Adolescent

Inpatient Treatment Alternatives

Eating Disorders, Inpatient Behavioral Health Level of Care, Child or Adolescent BHG

- Admission to **Inpatient Level of Care for Eating Disorder for Child or Adolescent** (CALOCUS-CASII Level 6 - Medically Managed Residence Based Services, Composite Score 28 or more) is indicated due to **ALL** of the following[A][B][C][D](4)(6)(13)(14)(15)(16)(17)(18)(19)(20)(21):
 - Patient risk and clinical condition are appropriate for inpatient treatment, as indicated by **1 or more** of the following(22):
 - ☐ Subnormal BMI or low expected body weight for height, age, and sex, and need for medical treatment of unstable physical condition and urgent refeeding are present, as indicated by **1 or more** of the following[E](23)(24)(25) BMI Calculator:
 - Current rapid rate of weight loss that has created an unstable physical condition
 - Core body temperature less than 96 degrees F (35.6 degrees C)(26)
 - Dehydration that is severe or persistent
 - Heart rate less than 50 beats per minute daytime or less than 45 beats per minute nighttime(26)
 - Hypotension
 - Orthostatic hypotension not responsive to appropriate

Eating Disorders, Residential Behavioral Health Level of Care, Child or Adolescent BHG

- Admission to **Residential Level of Care for Eating Disorder for Child or Adolescent** (CALOCUS-CASII Level 5 - Medically Monitored Residence Based Services, Composite Score 23 to 27) is indicated due to **ALL** of the following[C][I][U][V](4)(5)(6)(13)(14)(15)(16)(17)(20)(22)(26)(29)(41):
 - Strict staff supervision of all meals and bathroom use is necessary (eg, to curb pathologic weight-related behaviors, such as restrictive eating).
 - Motivation to recover is poor to fair (patient requires around-the-clock highly structured treatment setting to promote cooperation with care and weight gain).[I]
 - ☐ Behaviors or clinical findings (eg, weight gain pattern, food refusal, purging, medication use for weight control) are appropriate for residential level of care, as indicated by **1 or more** of the following[W]:
 - There has been sustained inability to achieve or maintain clinically appropriate weight goals.[X]
 - There has been continued or renewed compensatory

Observation Behavioral Health Level of Care BHG

- Admission to **Observation Level of Care** is indicated due to **ALL** of the following[C](9)(42)(43)(44)(45):
 - Care needed requires further evaluation, treatment, or disposition arrangement for behavioral health issues, as indicated by **1 or more** of the following(38)(46)(47)(48):
 - Imminent danger to self for adult or Imminent danger to self for child or adolescent
 - Imminent danger to others for adult or Imminent danger to others for child or adolescent
 - **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent
 - Patient is experiencing withdrawal or other drug-induced disorder. [F][G](8)(54)(55)(56)(57)(58) CIWA-Ar Calculator COWS Calculator
 - ☐ Observation care is appropriate for anticipated scope and care needed for patient, as indicated by **ALL** of the following:
 - Care is beyond scope of usual outpatient care episode or symptoms are not adequately controlled

Crisis Intervention Behavioral Health Level of Care BHG

- Admission to **Crisis Intervention Level of Care** (LOCUS/CALOCUS-CASII Basic Services) is indicated due to **ALL** of the following[C][J](62)(65)(66)(67)(68)(69)(70):
 - Around-the-clock behavioral care or close monitoring and intervention is indicated by **1 or more** of the following(32)(71)(72):
 - Danger to self for adult or Danger to self for child or adolescent
 - Danger to others for adult or Danger to others for child or adolescent
 - **Serious** dysfunction in daily living for adult or **Serious** dysfunction in daily living for child or adolescent
 - ☐ Patient management/treatment at lower level of care is not feasible (eg, lower level of care is unavailable or is not appropriate for patient condition) until acute intervention or modification (eg, acute

outpatient treatment (eg, hydration) • Prolonged corrected QT interval[F] • Severe muscle weakness[G] • Serum phosphorus less than 1.5 mg/dL (0.48 mmol/L)[H] • Electrolyte abnormality that cannot be corrected (to near normal) in emergency department or other ambulatory setting (eg, serum potassium less than 2.5 mEq/L (mmol/L), serum sodium less than 130 mEq/L (mmol/L)) • Significant injury due to purging (eg, mucosal (Mallory-Weiss) tear, hematemesis due to ongoing frequent vomiting, colonic injury due to enema misuse) • Malnutrition-related severe organ dysfunction or damage findings (eg, heart failure, arrhythmia, altered mental status) ■ Imminent risk of developing significant medical instability (eg, marked vital sign abnormalities, malnourishment requiring refeeding) due to rapid rate of weight loss(23) ☐ Supervisory needs, motivation to recover, weight-related behaviors, and comorbidities are appropriate for inpatient treatment, as indicated by ALL of the following: • Strict staff supervision of meals (may include monitoring of specialized feeding modality, such as nasogastric tube) and bathroom use (direct monitoring in bathroom) is necessary.(29) • Motivation to recover is very poor to poor (patient condition requires involuntary treatment, or if voluntary patient, highly structured, inpatient setting is necessary for adherence to care).[I] ☐ Behaviors or clinical findings (eg, weight gain	weight-loss behavior (eg, food refusal, self-induced vomiting, or excessive exercise). [L][Y][Z] ■ There has been continued or renewed use of pharmaceuticals with intent to control weight (eg, laxatives, diuretics, stimulants, or over-the-counter weight loss preparations). [Z] ☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following[M]: ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for residential care for child or adolescent, as indicated by ALL of the following[AA] (1)(2)(3)(5)(10)(15)(30): ■ Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, less intensive level is unavailable or not suitable for patient condition or history). [BB] ■ Very short-term crisis intervention and resource planning for further care at nonresidential level is unavailable or inappropriate. ■ Patient is willing to participate in treatment within highly structured setting voluntarily (or agrees to participate at	despite emergency department care.[HH] ■ Care needed is appropriate for observation care, as indicated by 1 or more of the following(42) (46)(59): • Diagnostic evaluation is needed and is not feasible in outpatient setting (eg, awaiting psychiatric evaluation which is not available in ambulatory care, or outpatient evaluation is inappropriate to patient condition). • Acute treatment and response evaluation is needed (eg, acute medication and observation for exacerbation of chronic psychosis). • Monitoring is needed while awaiting suitability for evaluation (eg, patient cannot be adequately assessed because of acute intoxication or substance-induced effect) or psychosocial management and arrangement for follow-up care. ■ Biopsychosocial stressors[O] potentially contributing to clinical presentation (eg, comorbidities,[P][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[II]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(5)(7)(8)(9) (10)(12)(39)	medication, identification of needed resources and support, change in living situation) is initiated.[KK] ☐ Situation and expectations are appropriate for crisis intervention services, as indicated by ALL of the following(7)(8)(31)(37): ■ Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, lower level of care is inappropriate for patient condition or is not available). [LL] ■ Sufficient treatment outcome (eg, stabilization and identification of resources and support for care outside of crisis intervention services) is expected within short time period (eg, 1 to 3 days). ■ Patient is willing to participate in treatment (or agrees to participate at direction of parent or guardian) within specified intervention and treatment structure voluntarily (or due to court order).[MM][NN] ■ There is no anticipated need for physical restraint, seclusion, or other involuntary control (eg, patient not actively violent). ■ There is no need for around-the-clock medical or nursing care. ■ Patient has sufficient ability to respond as planned to individual and group therapeutic interventions. [OO]
--	--	---	--

pattern, food refusal, purging, medication use for weight control) are appropriate for inpatient level of care, as indicated by **1 or more** of the following^[J](29):

- o There has been sustained inability to achieve or maintain clinically appropriate weight goals.
- o There has been continued or renewed compensatory weight-loss behavior (eg, food refusal, self-induced vomiting, or excessive exercise).^{[K][L]}
- o There has been continued or renewed use of pharmaceuticals with intent to control weight (eg, laxatives, diuretics, stimulants, or over-the-counter weight loss preparations).^[K]

☐ Treatment services available at proposed level of care are necessary to meet patient needs and **1 or more** of the following^[M](20)(21):

- Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care.
- Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care.
- Patient is receiving continuing care (eg, transition of care from less intensive level of care).

☐ Situation and expectations are appropriate for inpatient care for child or adolescent, as indicated by **1 or more** of the following⁽¹⁾⁽²⁾⁽³⁾⁽⁵⁾⁽¹⁰⁾⁽³⁰⁾⁽³¹⁾⁽³²⁾:

- Patient is unwilling to participate voluntarily in treatment and requires treatment (eg, legal commitment or order by guardian) in an involuntary unit.
- Voluntary treatment at lower level of care is not feasible (eg, very short-term crisis intervention or residential care

direction of parent or guardian, or attend due to court order).^{[CC][DD]}

- There is no anticipated need for physical restraint, seclusion, or other involuntary treatment interventions (eg, patient not actively violent).
- Medical or nursing care services to address primary admission diagnosis are available, as indicated by **1 or more** of the following:
 - No anticipated need for around-the-clock medical or nursing monitoring (ie, comorbid medical, psychiatric, or behavioral conditions are absent or are of minimal severity, and are not expected to interfere with recovery)
 - Active (but not around-the-clock) monitoring of patient by staff needed, and medical or nursing care can easily be provided if need arises (ie, comorbid medical, psychiatric, or behavioral conditions have potential to distract from treatment)
 - Around-the-clock medical or

- Biopsychosocial stressors^[O] potentially contributing to clinical presentation (eg, comorbidities, ^{[P][Q]} illness history, environment,^[R] social network, ability to cope, and level of engagement^[III]) have been assessed and are absent or manageable at proposed level of care.⁽¹⁾⁽²⁾⁽³⁾⁽⁵⁾⁽⁷⁾⁽⁸⁾⁽⁹⁾⁽¹⁰⁾⁽¹²⁾⁽³⁹⁾

- unavailable or insufficient for patient condition).

■ Need for physical restraint, seclusion, or other involuntary treatment intervention is present (eg, actively violent patient for whom treatment in an involuntary unit is deemed necessary in accordance with applicable medical and legal criteria).

■ Around-the-clock medical and nursing care to address symptoms and initiate intervention is required; specific need is identified.[N]

■ Patient management/treatment at lower level of care is not feasible or is inappropriate (eg, less intensive level of care is unavailable or not suitable for patient condition or treatment history).

■ Biopsychosocial stressors[O] potentially contributing to clinical presentation (eg, comorbidities,[P][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[S]) have been assessed and are absent or manageable at proposed level of care.(1)(2)(3)(5)(7)(8)(9)(10)(12)(39)
- nursing monitoring needed, but intensive treatment and resources of licensed hospital are not anticipated (ie, due to severity of primary admitting diagnosis, or presence of active comorbid medical, psychiatric, or behavioral conditions that are distracting from treatment)

■ Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.[EE]

■ Biopsychosocial stressors[O] potentially contributing to clinical presentation (eg, comorbidities, [P][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[S]) have been assessed and are absent or manageable at proposed level of care.(1)(2)(3)(5)(7)(8)(9)(10)(12)(39)

Outpatient Treatment Alternatives

- Eating Disorders, Partial Hospital Behavioral Health Level of Care, Child or Adolescent [BHG](#)

• Admission to **Partial Hospital Level of Care for Eating Disorder for Child or Adolescent** (CALOCUS-CASII Level 4 - Medically Monitored Community Based Services, Composite Score 20 to 22) is indicated due to **ALL** of the following[C][PP][QQ][RR](4)(6)(13)(14)(16)(17)(20)(21)(22)(26)(41)(75)(76)(77):
 - Daily (or near daily) staff supervision of meals or

Eating Disorders, Intensive Outpatient Program Behavioral Health Level of Care, Child or Adolescent [BHG](#)

• Admission to **Intensive Outpatient Program Level of Care for Eating Disorder for Child or Adolescent** (CALOCUS-CASII Level 3 - High Intensity Community Based Services, Composite Score 17 to 19) is indicated due to **ALL** of the following[C][DD](17)(20)(21)(26)(41):

Eating Disorders, Outpatient Behavioral Health Level of Care, Child or Adolescent [BHG](#)

• Admission to **Outpatient Level of Care for Eating Disorder for Child or Adolescent** (CALOCUS-CASII Level 2 - Low Intensity Community Based Services, Composite Score 14 to 16) is indicated due to **ALL** of the following[C][KK][LLL][RR](4)(5)(6)(13)(14)(15)(16)(17)(20)(21)(26)(41):
 - Supervision of meals and bathroom use is not necessary.(29)

Day Treatment Behavioral Health Level of Care [BHG](#)

• Admission to **Day Treatment Level of Care** is indicated due to **ALL** of the following[C][QQQ][RRR]:
 - Behavioral health disorder is present and patient symptom severity and functional status is appropriate to proposed level of care, as indicated by **ALL** of the following[SSS][TTT](7)(8)(9)(12)(37)(64):

https://careweb.careguidelines.com/ed28/bhg/bhg_04754.htm?printview=1

4/37

bathroom use (5 to 7 days a week) is necessary to ensure adequate intake and prevent bingeing or purging.[SS](12)(29) - o Motivation to recover is partially impaired and appropriate for partial hospital care (daily or near daily structured program is necessary to promote cooperation and weight gain).[I](12)(29) ☐ Behaviors or clinical findings (eg, weight gain pattern, food refusal, purging, medication use for weight control) are appropriate for partial hospital level of care, as indicated by 1 or more of the following[TT](12)(29): ■ There has been sustained inability to achieve or maintain clinically appropriate weight goals.[X] ■ There has been continued or renewed compensatory weight-loss behavior (eg, food refusal, self-induced vomiting, or excessive exercise).[L][UU][VV] ■ There has been continued or renewed use of pharmaceuticals with intent to control weight (eg, laxatives, diuretics, stimulants, or over-the-counter weight loss preparations).[VV] ☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following[M](29)(75): ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for partial hospital program for child or adolescent, as indicated by	- o Supervision of meals and bathroom use is not necessary.(29) - o Motivation to recover is assessed as fair (attendance at program 1 to 2 times a week generally is sufficient to promote cooperation with care and weight gain).[EEE](12)(29) ☐ Behaviors or clinical findings (eg, weight gain pattern, food refusal, purging, medication use for weight control) are appropriate for intensive outpatient level of care, as indicated by 1 or more of the following[FFF](29): ■ There has been sustained inability to achieve or maintain clinically appropriate weight goals.[X] ■ There has been continued or renewed compensatory weight-loss behavior (eg, food refusal, self-induced vomiting, or excessive exercise).[GGG][HHH][L] ■ There has been continued or renewed use of pharmaceuticals with intent to control weight (eg, laxatives, diuretics, stimulants, over-the-counter weight loss preparations).[HHH] ☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following[M](20)(21): ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for intensive outpatient program for child or adolescent, as indicated by ALL of the following[III][XX][YY]	- o Motivation to recover or maintain remission is assessed as fair to good and appropriate for outpatient care (structured program is not necessary to promote cooperation with care).[MMM](29) ☐ Behaviors or clinical findings (eg, weight gain pattern, food refusal, purging, medication use for weight control) are appropriate for outpatient care (ie, structured setting is not required for management), as indicated by 1 or more of the following(29): ■ There has been sustained inability to achieve or maintain clinically appropriate weight goals.[X] ■ There has been continued or renewed compensatory weight-loss behavior (eg, food refusal, self-induced vomiting, or excessive exercise).[NNN][OOO] ■ There has been continued or renewed use of pharmaceuticals with intent to control weight (eg, laxatives, diuretics, stimulants, over-the-counter weight loss preparations).[NNN] ☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following[M](20)(21)(26): ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for outpatient care for child or adolescent, as indicated by ALL of the following[PPP](1)(2)(3)(5)(15)	■ Mild to moderate Psychiatric, behavioral, or other comorbid conditions for adult or Mild to moderate Psychiatric, behavioral, or other comorbid conditions for child or adolescent [UUU][VVV][WWW] ■ Mild to moderate dysfunction in daily living for adult or Mild to moderate dysfunction in daily living for child or adolescent ☐ Patient condition does not require urgent intervention, as indicated by ALL of the following[XXX]: ■ Signs or symptoms related to admitting diagnosis (or impact of comorbidity on admitting diagnosis) are stable or improving and there has not been an acute increase in condition severity. ■ Impairments in function are stable or improving and there has not been a recent substantial deterioration in function. ☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following[YYY]: ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for day treatment services, as indicated by ALL of the following(5)(7)(8)(37): ■ Recommended treatment is necessary and appropriate[ZZZ] and not feasible with less
--	--	---	---

ALL of the following[WW][XX]

[YY](1)(2)(3)(5)(10)(12)(15)(30):

- Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, less intensive level is unavailable or not suitable to patient condition or history). [ZZ]
- Patient is willing to participate in treatment voluntarily (or at direction of parent or guardian). [CC][AAA][DD]
- There is no anticipated need for physical restraint or other involuntary treatment interventions.
- There is no need for around-the-clock medical or nursing care.
- There has been no recent attempt or Current plan to seriously Harm another.
- There has been no recent Suicide attempt or act of serious Harm to self, or there has been Sufficient relief of precipitants of any such action.
- There is no Current plan for suicide or serious Harm.
- Patient's family, caregiver, guardian, or other available resources can adequately monitor patient condition when away from partial hospital program (ie, partial hospital program resources are sufficient to address any inadequacies in support system). [BBB]
- Patient, family, caregiver, or guardian will contact care provider if Current plan for suicide or serious Harm develops. [AAA] (12)(78)
- Patient has sufficient ability to respond as planned to individual and group therapeutic interventions. [CCC] [EE]

(1)(2)(3)(5)(10)(15)(30):

- Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, less intensive level is unavailable or not suitable to patient condition or history). [ZZ]
- Patient is willing to participate in treatment voluntarily, or at least agrees to participate at direction of parent or guardian. [AAA][CC] [DD]
- Patient has sufficient ability to respond as planned to individual and group therapeutic interventions. [CCC] [EE]
- Biopsychosocial stressors [JJJ] potentially contributing to clinical presentation (eg, comorbidities, [P] [Q] illness history, environment, [R] social network, ability to cope, and level of engagement [S]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(5)(7) (8)(9)(10)(12)(39)

(30):

- Recommended treatment is necessary and appropriate, given patient condition or history. [BB]
- Patient is willing to participate in treatment voluntarily (or agrees to participate at direction of parent or guardian). [CC][DD]
- Patient has sufficient ability to respond as planned to individual and group therapeutic interventions. [EE]
- Biopsychosocial stressors [O] potentially contributing to clinical presentation (eg, comorbidities, [P] [Q] illness history, environment, [R] social network, ability to cope, and level of engagement [S]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(5)(7) (8)(9)(10)(12)(39)

intensive intervention

- (eg, less intensive support, such as outpatient care, is not suitable to patient condition or history). [LL]
- Patient and family or caregivers, as appropriate, are willing to participate in treatment voluntarily. [MM][NN]
 - Patient has sufficient ability to respond as planned to individual and group therapeutic interventions. [OO] (94)(95)
 - Biopsychosocial stressors [O] potentially contributing to clinical presentation (eg, comorbidities, [P] [Q] illness history, environment, [R] social network, ability to cope, and level of engagement [I]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(5)(7) (8)(9)(10)(12)(39)

- Biopsychosocial stressors^[O] potentially contributing to clinical presentation (eg, comorbidities,^[P] ^[Q] illness history, environment,^[R] social network, ability to cope, and level of engagement^[S]) have been assessed and are absent or manageable at proposed level of care.⁽¹⁾⁽²⁾⁽³⁾⁽⁵⁾⁽⁷⁾⁽⁸⁾⁽⁹⁾⁽¹⁰⁾⁽¹²⁾⁽³⁹⁾

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, 30, 31, 32, 33, 34, 35, 36, 37, 38, 39, 40, 41, 42, 43, 44, 45, 46, 47, 48]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, 30, 31, 32, 33, 34, 35, 36, 37, 38, 39, 40, 41, 42, 43, 44, 45, 46, 47, 48]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, 30, 31, 32, 33, 34, 35, 36, 37, 38, 39, 40, 41, 42, 43, 44, 45, 46, 47, 48]
4. Call CC, Attia E, Walsh BT. Feeding and eating disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2065-2082. [Context Link 1, 2, 3, 4, 5, 6, 7]
5. Kober D, Martin A. Inpatient psychiatric, partial hospital, and residential treatment for children and adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3797-3803. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23]
6. Crone C, et al. The American Psychiatric Association Practice Guideline for the Treatment of Patients With Eating Disorders. 4th ed [Internet] American Psychiatric Association. 2023 Feb Accessed at: http://www.psych.org/psych_pract/treatg/pg/prac_guide.cfm. [accessed 2023 Sep 19] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, 30, 31, 32, 33]
7. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, 30, 31, 32, 33]
8. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21]
9. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16]
10. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16]
11. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857. [Context Link 1]
12. Rosser J, Michael S. Partial Hospitalization Programs and Intensive Outpatient Programs. 2021 AABH Standards and Guidelines [Internet] Association for Ambulatory Behavioral Healthcare. 2021 Accessed at: <https://aabh.org/>. [accessed 2023 Sep 12] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18, 19, 20, 21, 22, 23, 24, 25, 26, 27, 28, 29, 30, 31, 32]
13. Hay P. Current approach to eating disorders: a clinical update. Internal Medicine Journal 2020;50(1):24-29. DOI: 10.1111/imj.14691. [Context Link 1, 2, 3, 4, 5] View abstract...
14. King RA, Fisher PW, Bloch MH, Thies AP, Schwab-Stone ME. Psychiatric examination of the infant, child, and adolescent. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3375-3409. [Context Link 1, 2, 3, 4, 5]
15. Schlozman SC, Beresin EV. The treatment of adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3810-3816. [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
16. Eating Disorders: Recognition and Treatment. NICE Guidance NG69 [Internet] National Institute for Health and Care Excellence. 2020 Dec Accessed at: <https://www.nice.org.uk/guidance>. [accessed 2023 Aug 08] [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
17. Feeding and eating disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:371-397. [Context Link 1, 2, 3, 4, 5]
18. Gowers SG, et al. A randomised controlled multicentre trial of treatments for adolescent anorexia nervosa including assessment of cost-effectiveness and patient acceptability - the TOUCAN trial. Health Technology Assessment 2010;14(15):1-98. DOI: 10.3310/hta14150. [Context Link 1] View abstract...
19. Suarez-Pinilla P, et al. Inpatient treatment for anorexia nervosa: a systematic review of randomized controlled trials. Journal of Psychiatric Practice 2015;21(1):49-59. DOI: 10.1097/01.pra.0000460621.95181.e2. [Context Link 1] View abstract...
20. Madden S, Hay P, Touyz S. Systematic review of evidence for different treatment settings in anorexia nervosa. World Journal of Psychiatry 2015;5(1):147-153. DOI: 10.5498/wjp.v5.i1.147. [Context Link 1, 2, 3, 4, 5, 6, 7, 8] View abstract...
21. Clausen L, Jones A. A systematic review of the frequency, duration, type and effect of involuntary treatment for people with anorexia nervosa, and an analysis of patient characteristics. Journal of Eating Disorders 2014;2(1):29. DOI: 10.1186/s40337-014-0029-8. [Context Link 1, 2, 3, 4, 5, 6, 7] View abstract...
22. Johnson MR, Hatzis NM, Jutla A. Children and adolescents. In: Roberts LW, editor. The American Psychiatric Publishing Textbook of Psychiatry. 7th ed. Washington DC: American Psychiatric Association Publishing; 2019:1147-1184. [Context Link 1, 2, 3]
23. Hilbert A, Hoek HW, Schmidt R. Evidence-based clinical guidelines for eating disorders: international comparison. Current Opinion in Psychiatry 2017;30(6):423-437. DOI: 10.1097/YCO.0000000000000360. [Context Link 1, 2, 3, 4] View abstract...

24. Mehler PS, Brown C. Anorexia nervosa - medical complications. *Journal of Eating Disorders* 2015;3:11. DOI: 10.1186/s40337-015-0040-8. [Context Link 1] View abstract...
25. Society for Adolescent Health and Medicine, et al. Position Paper of the Society for Adolescent Health and Medicine: medical management of restrictive eating disorders in adolescents and young adults. *Journal of Adolescent Health* 2015;56(1):121-5. DOI: 10.1016/j.jadohealth.2014.10.259. [Context Link 1] View abstract...
26. Golden NH, et al. Update on the medical management of eating disorders in adolescents. *Journal of Adolescent Health* 2015;56(4):370-5. DOI: 10.1016/j.jadohealth.2014.11.020. [Context Link 1, 2, 3, 4, 5, 6, 7, 8] View abstract...
27. Sachs KV, Harnke B, Mehler PS, Krantz MJ. Cardiovascular complications of anorexia nervosa: A systematic review. *International Journal of Eating Disorders* 2016;49(3):238-48. DOI: 10.1002/eat.22481. [Context Link 1] View abstract...
28. Sachs K, Andersen D, Sommer J, Winkelman A, Mehler PS. Avoiding medical complications during the refeeding of patients with anorexia nervosa. *Eating Disorders* 2015;1-11. DOI: 10.1080/10640266.2014.1000111. [Context Link 1] View abstract...
29. Levels of Care. [Internet] National Eating Disorders Association (NEDA). Accessed at: <https://www.nationaleatingdisorders.org/types-treatment>. Updated 2022 [accessed 2023 Oct 23] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16]
30. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150. [Context Link 1, 2, 3, 4, 5, 6]
31. Baron DA, Cobb RT, Juarez GM. Other psychiatric emergencies. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2622-2637. [Context Link 1, 2]
32. Petti TA. Milieu treatment: inpatient, partial hospitalization, and residential programs. In: Dulcan MK, editor. *Dulcan's Textbook of Child and Adolescent Psychiatry*. 3rd ed. American Psychiatric Association Publishing; 2022:1019-1042. [Context Link 1, 2]
33. Abadi M, et al. Achieving whole health: a preliminary study of TCMLH, a group-based program promoting self-care and empowerment among Veterans. *Health Education & Behavior* 2022;49(2):347-357. DOI: 10.1177/10901981211011043. [Context Link 1, 2, 3, 4] View abstract...
34. Joshi H, Corcoran-Lozano P. The behavioral health consultant: roles and responsibilities. *Pediatric Clinics of North America* 2021;68(3):563-571. DOI: 10.1016/j.pcl.2021.02.002. [Context Link 1, 2, 3, 4] View abstract...
35. Rosenbaum E, Gordon AE, Cresta J, Shaughnessy AF, Jonas WB. Implementing whole person primary care. *Annals of Family Medicine* 2023;21(2):Online. DOI: 10.1370/afm.2952. [Context Link 1, 2, 3, 4] View abstract...
36. Pourat N, et al. Final Evaluation of California's Whole Person Care (WPC) Program. [Internet] UCLA Center for Health Policy Research. 2022 Dec Accessed at: <https://healthpolicy.ucla.edu/publications/Pages/default.aspx>. [accessed 2023 Sep 08] [Context Link 1, 2, 3, 4]
37. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11]
38. Service planning and placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:105-126. [Context Link 1, 2, 3]
39. Fong T. Assessment. In: Brady KT, Levin FR, Galanter M, Kleber HD, editors. *Textbook of Substance Use Disorder Treatment*. 6th ed. American Psychiatric Association Publishing; 2021:63-76. [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
40. Houston M, et al. Principles of Care for Treatment of Children and Adolescents with Mental Illnesses in Residential Treatment Centers. [Internet] American Academy of Child and Adolescent Psychiatry. 2010 Jun Accessed at: <https://www.aacap.org/>. [accessed 2023 Sep 26] [Context Link 1]
41. Attia E. Eating disorders. *Annals of Internal Medicine* 2012;156(7):ITC41. DOI: 10.1059/0003-4819-156-7-201204030-01004. [Context Link 1, 2, 3, 4] View abstract...
42. Kelly MP, Shapshak D. Thought disorders. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1333-1337.e2. [Context Link 1, 2, 3]
43. Iannone P, Lenzi T. Effectiveness of a multipurpose observation unit: before and after study. *Emergency Medicine Journal* 2009;26(6):407-14. DOI: 10.1136/emj.2007.057539. [Context Link 1] View abstract...
44. Alpern ER, Calello DP, Windreich R, Osterhoudt K, Shaw KN. Utilization and unexpected hospitalization rates of a pediatric emergency department 23-hour observation unit. *Pediatric Emergency Care* 2008;24(9):589-94. [Context Link 1] View abstract...
45. Soyka M, et al. The World Federation of Societies of Biological Psychiatry (WFSBP) guidelines for the biological treatment of substance use and related disorders. Part 2: Opioid dependence. *World Journal of Biological Psychiatry* 2011;12(3):160-187. DOI: 10.3109/15622975.2011.561872. (Reaffirmed 2022 May) [Context Link 1] View abstract...
46. Leykum LK, Huerta V, Mortensen E. Implementation of a hospitalist-run observation unit and impact on length of stay (LOS): a brief report. *Journal of Hospital Medicine* 2010 Nov-Dec;5(9):E2-5. DOI: 10.1002/jhm.642. [Context Link 1, 2] View abstract...
47. Young HW, Shapiro MA. Suicidal Behavior. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1358-1365.e4. [Context Link 1]
48. Abraham G, Maher PJ. Delirium and dementia. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1269-1280.e2. [Context Link 1]
49. Nelson LS, et al. The ASAM Clinical Practice Guideline on Alcohol Withdrawal Management. [Internet] American Society of Addiction Medicine. 2020 Jan Accessed at: <https://www.asam.org/>. [accessed 2023 Oct 09] [Context Link 1]
50. Sullivan JT, Sykora K, Schneiderman J, Naranjo CA, Sellers EM. Assessment of alcohol withdrawal: the revised clinical institute withdrawal assessment for alcohol scale (CIWA-Ar). *British Journal of Addiction* 1989;84(11):1353-1357. [Context Link 1] View abstract...
51. Clinical Institute Withdrawal Assessment for Alcohol (CIWA-Ar). [Internet] Merck Manual. Accessed at: <https://www.merckmanuals.com/professional/multimedia/clinical-calculator/ciwa-ar-clinical-institute-withdrawal-assessment-for-alcohol-scale>. [accessed 2023 Oct 20] [Context Link 1]
52. Wesson DR, Ling W. The Clinical Opiate Withdrawal Scale (COWS). *Journal of Psychoactive Drugs* 2003 Apr-Jun;35(2):253-259. DOI: 10.1080/02791072.2003.10400007. [Context Link 1] View abstract...
53. Clinical Opiate Withdrawal Scale. [Internet] National Institutes of Health National Institute on Drug Abuse. 2003 Accessed at: <https://www.drugabuse.gov/nidamed-medical-health-professionals>. [created 1999; accessed 2023 Oct 17] [Context Link 1]
54. Finnell JT. Alcohol-related disease. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1846-1860.e2. [Context Link 1]
55. Addressing withdrawal management and intoxication management. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:127-173. [Context Link 1]
56. Kleber HD, et al. Practice Guideline for the Treatment of Patients with Substance Use Disorders. 2nd ed. [Internet] American Psychiatric Association. 2007 Apr Accessed at: <https://psychiatryonline.org/>. [created 2006; accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890423363.141077. [Context Link 1]
57. Strain EC. Substance use disorders: introduction. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1262-1264. [Context Link 1]
58. Drug Misuse Opioid Detoxification. NICE Clinical Guidance CG52 [Internet] National Institute for Health and Care Excellence. 2007 Jul (NICE reviewed 2019) Accessed at: <https://www.nice.org.uk/guidance>. [accessed 2023 Aug 08] [Context Link 1] View abstract...
59. Ross MA, Graff LGt. Principles of observation medicine. *Emergency Medicine Clinics of North America* 2001;19(1):1-17. [Context Link 1, 2] View abstract...
60. Venkatesh AK, Geisler BP, Gibson Chambers JJ, Baugh CW, Bohan JS, Schuur JD. Use of observation care in US emergency departments, 2001 to 2008. *PLoS ONE* 2011;6(9):e24326. DOI: 10.1371/journal.pone.0024326. [Context Link 1] View abstract...

61. Lloyd-Evans B, Slade M, Jagielska D, Johnson S. Residential alternatives to acute psychiatric hospital admission: systematic review. *British Journal of Psychiatry* 2009;195(2):109-17. DOI: 10.1192/bjp.bp.108.058347. [Context Link 1, 2] View abstract...
62. Murphy SM, Irving CB, Adams CE, Waqar M. Crisis intervention for people with severe mental illnesses. *Cochrane Database of Systematic Reviews* 2015, Issue 12. Art. No.: CD001087. DOI: 10.1002/14651858.CD001087.pub5. [Context Link 1, 2] View abstract...
63. Vandevooren J, Miller L, O'Reilly R. Outcomes in community-based residential treatment and rehabilitation for individuals with psychiatric disabilities: a retrospective study. *Psychiatric Rehabilitation Journal* 2007;30(3):215-7. [Context Link 1] View abstract...
64. Marshall M, Crowther R, Sledge WH, Rathbone J, Soares-Weiser K. Day hospital versus admission for acute psychiatric disorders. *Cochrane Database of Systematic Reviews* 2011, Issue 12. Art. No.: CD004026. DOI: 10.1002/14651858.CD004026.pub2. [Context Link 1, 2, 3] View abstract...
65. Fenton WS, Hoch JS, Herrell JM, Mosher L, Dixon L. Cost and cost-effectiveness of hospital vs residential crisis care for patients who have serious mental illness. *Archives of General Psychiatry* 2002;59(4):357-64. [Context Link 1] View abstract...
66. Berrino A, Ohlendorf P, Duriaux S, Burnand Y, Lorillard S, Andreoli A. Crisis intervention at the general hospital: an appropriate treatment choice for acutely suicidal borderline patients. *Psychiatry Research* 2011;186(2-3):287-92. DOI: 10.1016/j.psychres.2010.06.018. [Context Link 1] View abstract...
67. Cotton MA, et al. An investigation of factors associated with psychiatric hospital admission despite the presence of crisis resolution teams. *BMC Psychiatry* 2007;7:52. DOI: 10.1186/1471-244X-7-52. [Context Link 1, 2] View abstract...
68. Adams CL, El-Mallakh RS. Patient outcome after treatment in a community-based crisis stabilization unit. *Journal of Behavioral Health Services & Research* 2009;36(3):396-9. DOI: 10.1007/s11414-008-9141-3. [Context Link 1] View abstract...
69. Howard LM, Rigon E, Cole L, Lawlor C, Johnson S. Admission to women's crisis houses or to psychiatric wards: women's pathways to admission. *Psychiatric Services* 2008;59(12):1443-9. DOI: 10.1176/appi.ps.59.12.1443. [Context Link 1] View abstract...
70. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15] [Context Link 1]
71. Gelenberg AJ, et al. Treatment of Patients With Major Depressive Disorder. 3rd edition [Internet] American Psychiatric Association. 2010 Nov Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890423387.654001. [Context Link 1]
72. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622. [Context Link 1]
73. Management of Substance Use Disorders Work Group. VA/DoD Clinical Practice Guideline for the Management of Substance Use Disorders. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. 2021 Accessed at: <https://www.healthquality.va.gov/>. [created 2004; accessed 2023 May 15] [Context Link 1, 2]
74. Olmsted MP, McFarlane T, Trottier K, Rockert W. Efficacy and intensity of day hospital treatment for eating disorders. *Psychotherapy Research* 2013;23(3):277-86. DOI: 10.1080/10503307.2012.721937. [Context Link 1] View abstract...
75. Herpertz-Dahlmann B, et al. Day-patient treatment after short inpatient care versus continued inpatient treatment in adolescents with anorexia nervosa (ANDI): a multicentre, randomised, open-label, non-inferiority trial. *Lancet* 2014;383(9924):1222-1229. DOI: 10.1016/S0140-6736(13)62411-3. [Context Link 1, 2, 3] View abstract...
76. Fittig E, Jacobi C, Backmund H, Gerlinghoff M, Wittchen HU. Effectiveness of day hospital treatment for anorexia nervosa and bulimia nervosa. *European Eating Disorders Review* 2008;16(5):341-351. DOI: 10.1002/erv.883. [Context Link 1] View abstract...
77. Degraft-Johnson A, Fisher M, Rosen L, Napolitano B, Laskin E. Weight gain in an eating disorders day program. *International Journal of Adolescent Medicine and Health* 2013;25(2):177-180. DOI: 10.1515/ijamh-2013-0027. [Context Link 1] View abstract...
78. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. *Pediatrics* 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul) [Context Link 1] View abstract...
79. Garrison D, Daigler GE. Treatment settings for adolescent psychiatric conditions. *Adolescent Medicine Clinics* 2006;17(1):233-50. DOI: 10.1016/j.admecli.2005.10.002. [Context Link 1, 2, 3, 4] View abstract...
80. Sussman S. A review of Alcoholics Anonymous/ Narcotics Anonymous programs for teens. *Evaluation & the Health Professions* 2010;33(1):26-55. DOI: 10.1177/0163278709356186. [Context Link 1, 2] View abstract...
81. Vanderploeg JJ, Franks RP, Plant R, Cloud M, Tebes JK. Extended day treatment: a comprehensive model of after school behavioral health services for youth. *Child and Youth Care Forum* 2010;38(1):5-18. DOI: 10.1007/s10566-008-9062-6. [Context Link 1, 2] View abstract...
82. Trask EV, Fawley-King K, Garland AF, Aarons GA. Do aftercare mental health services reduce risk of psychiatric rehospitalization for children? *Psychological Services* 2016;13(2):127-32. DOI: 10.1037/ser0000043. [Context Link 1] View abstract...
83. Sullivan E, Fleming M. A Guide to Substance Abuse Services for Primary Care Clinicians. Treatment Improvement Protocol (TIP) Series, No. 24 [Internet] Substance Abuse and Mental Health Services Administration. 1997 Accessed at: <https://www.ncbi.nlm.nih.gov/books/NBK64827/>. [accessed 2023 Aug 15] [Context Link 1] View abstract...
84. Bukumiric Z, et al. Meta-analysis of the changes in correlations between depression instruments used in longitudinal studies. *Journal of Affective Disorders* 2016;190:733-743. DOI: 10.1016/j.jad.2015.10.054. [Context Link 1] View abstract...
85. Kroenke K, Spitzer RL, Williams JB. The PHQ-9: validity of a brief depression severity measure. *Journal of General Internal Medicine* 2001;16(9):606-13. [Context Link 1, 2] View abstract...
86. Maurer DM, Raymond TJ, Davis BN. Depression: screening and diagnosis. *American Family Physician* 2018;98(8):508-515. [Context Link 1] View abstract...
87. US Preventive Services Task Force, et al. Screening for depression and suicide risk in children and adolescents: US Preventive Services Task Force recommendation statement. *Journal of the American Medical Association* 2022;328(15):1534-1542. DOI: 10.1001/jama.2022.16946. (Reaffirmed 2022 Oct) [Context Link 1, 2] View abstract...
88. Richardson LP, et al. Evaluation of the Patient Health Questionnaire-9 Item for detecting major depression among adolescents. *Pediatrics* 2010;126(6):1117-23. DOI: 10.1542/peds.2010-0852. [Context Link 1] View abstract...
89. Allgaier AK, Pietsch K, Fruhe B, Sigl-Glockner J, Schulte-Korne G. Screening for depression in adolescents: validity of the patient health questionnaire in pediatric care. *Depression and Anxiety* 2012;29(10):906-13. DOI: 10.1002/da.21971. [Context Link 1] View abstract...
90. Spitzer RL, Williams JB, Kroenke K. Patient Health Questionnaire-9 (PHQ-9). [Internet] Pfizer, Inc. Accessed at: <https://www.phqscreeners.com/>. [accessed 2023 Aug 10] [Context Link 1]
91. Riedel M, et al. Response and remission criteria in major depression--a validation of current practice. *Journal of Psychiatric Research* 2010;44(15):1063-8. DOI: 10.1016/j.jpsychires.2010.03.006. [Context Link 1] View abstract...
92. O'Haire ME. Animal-assisted intervention for autism spectrum disorder: a systematic literature review. *Journal of Autism and Developmental Disorders* 2013;43(7):1606-22. DOI: 10.1007/s10803-012-1707-5. [Context Link 1] View abstract...
93. Anestis MD, Anestis JC, Zawilinski LL, Hopkins TA, Lilienfeld SO. Equine-related treatments for mental disorders lack empirical support: a systematic review of empirical investigations. *Journal of Clinical Psychology* 2014;70(12):1115-32. DOI: 10.1002/jclp.22113. [Context Link 1] View abstract...
94. Grisso T, Appelbaum PS, Hill-Fotouhi C. The MacCAT-T: a clinical tool to assess patients' capacities to make treatment decisions. *Psychiatric Services* 1997;48(11):1415-9. [Context Link 1] View abstract...
95. Dunn LB, Nowrangi MA, Palmer BW, Jeste DV, Saks ER. Assessing decisional capacity for clinical research or treatment: a review of instruments. *American Journal of Psychiatry* 2006;163(8):1323-34. DOI: 10.1176/appi.ajp.163.8.1323. [Context Link 1] View abstract...

Footnotes

[A] Inpatient psychiatric units generally are locked, equipped to restrain or seclude patients for safety if necessary, and staffed by nurses around the clock. A covering physician is available to assess patients if indicated around the clock.(1)(2)(3)(4)(5) [A in Context Link 1]

[B] Whether or not a patient with an eating disorder should be hospitalized on a psychiatric vs a general medical or pediatric/adolescent unit depends on a variety of factors, including medical and psychiatric status, skills and abilities of staff in the proposed admitting institution, and the availability of suitable programs to address medical and psychiatric needs. There is some evidence to suggest that when hospital admission is necessary, admission to inpatient units specializing in eating disorder treatment may be associated with better outcomes.(6) [B in Context Link 1]

[C] The purpose of the care guidelines is to promote evidence-based care across the continuum of care to enhance the delivery of quality healthcare. Indications are presented for different levels of care. These indications help define the optimal level of care and can assist in developing alternatives to higher levels of care, tracking patient progress during treatment within a level of care, facilitating the progress of patients whose recovery is delayed, and preparing comprehensive plans for transition of patients from one level of care to another. Relevant professional society guidelines are foundational content for evaluation and treatment of behavioral health disorders at different levels of care and are complemented by the best available published evidence.(1)(2)(3)(7)(8)(9)(10)(11)(12) [C in Context Link 1, 2, 3, 4, 5, 6, 7, 8]

[D] Composite score does not replace clinical judgment and is meant as a guide to assist in determining needed level of care. Extreme risk of harm, severe functional impairment, or severe comorbidity may independently necessitate placement at inpatient level of care.(1)(2)(3) [D in Context Link 1]

[E] Practice guidelines for the treatment of patients with eating disorders indicate that the decision for hospitalization of a patient with an eating disorder should be made on an individualized basis, utilizing a patient's personal weight status and risk of developing medical complications, instead of using standardized cutoff levels for BMI, rate of weight change, or percentage of healthy body weight.(23) [E in Context Link 1]

[F] QT-interval prolongation is a possible complication of anorexia nervosa and a suspected contributor to the increased incidence of sudden death observed in patients with this disorder.(27) [F in Context Link 1]

[G] Muscle weakness is considered severe if it prevents the patient from rising from a seated position without use of the arms, or rising from a supine position even with use of the arms, or from being able to sit up at all.(16) [G in Context Link 1]

[H] Total body phosphorus depletion, a possible consequence of malnutrition, increases the risk (during nutritional rehabilitation) of refeeding syndrome, a condition involving severe alterations in fluid and electrolyte balances that can lead to potentially life-threatening cardiac and neuromuscular complications.(26)(28) [H in Context Link 1]

[I] A practice guideline for the treatment of patients with eating disorders indicates that an assessment of motivation to recover is indicated in the initial admission assessment to assist in determining the appropriate level of care to treat the eating disorder.(6) [I in Context Link 1, 2, 3]

[J] Patients appropriate for inpatient eating disorder treatment may be preoccupied with intrusive or repetitive thoughts for more than 6 hours a day.(6) [J in Context Link 1]

[K] A practice guideline for the treatment of patients with eating disorders indicates that patients engaged in and unable to control multiple daily episodes of purging behaviors that are severe, persistent, and disabling, and who have been unable to regulate these behaviors despite appropriate trials of outpatient care may be considered for inpatient management. This is the case even in the absence of metabolic abnormalities, provided that they otherwise meet inpatient admission criteria related to supervisory needs, motivation, and comorbidities.(6) [K in Context Link 1, 2]

[L] A practice guideline for the treatment of patients with eating disorders indicates that compulsive exercising rarely is a sole indication for increasing level of care.(6) [L in Context Link 1, 2, 3, 4]

[M] Assessment of the likelihood of benefit at the proposed level of care may help identify subgroups of patients who may be more likely to respond to particular treatments and help optimize the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or a different level of service intensity would be more appropriate to address the patient's needs.(1)(2)(3)(6)(8) [M in Context Link 1, 2, 3, 4, 5]

[N] Examples of medical conditions appropriate for inpatient care include conditions that require intensive, around-the-clock medical monitoring and daily nursing interventions, or patients with significant metabolic or ECG abnormalities related to vomiting.(1)(2)(3) [N in Context Link 1]

[O] Biopsychosocial stressors may impact the level of care necessary to manage a psychiatric or behavioral condition, including the ability of the program to meet comprehensive patient needs, ensure treatment adherence, enhance motivation, or prevent relapse (ie, comorbidities, environmental factors, or other barriers may prevent effective treatment at a less intensive level of care than might otherwise be appropriate to the patient's condition). Biopsychosocial assessment factors should be incorporated into care planning, including planned treatment goals, and intensity and duration of interventions. A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(33)(34)(35)(36) Any identified deficits should be manageable by the program directly or through alternative arrangements.(1)(2)(3)(7) [O in Context Link 1, 2, 3, 4, 5, 6, 7]

[P] Comorbid conditions may directly impact the experience of psychiatric symptoms (eg, COPD and anxiety), or may indirectly impact determination of the appropriate venue for care (eg, routine blood sugar and insulin management in people with diabetes). If clinically appropriate, testing related to diagnosis or management (eg, screening for liver and kidney function, hepatitis, HIV, syphilis, tuberculosis) may be performed off-site.(1)(2)(3)(7) Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(7)(9)(10) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(33)(34)(35)(36) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(1)(2)(3)(7)(37) [P in Context Link 1, 2, 3, 4, 5, 6, 7, 8]

[Q] Evaluation/assessment and treatment of co-occurring substance use disorders through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(7)(9) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(33)(34)(35)(36) The level that comorbid substance use disorders contribute to the primary presenting condition may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care or specialty dual-diagnosis program).(1)(2)(3)(7)(37) [Q in Context Link 1, 2, 3, 4, 5, 6, 7, 8]

[R] The degree of environmental stressors and amount of support in the patient recovery environment should be considered in the context of the clinical presentation in determining the appropriate level of care for treatment. The level of environmental stressors may be low, mild, moderate, high, or extreme. The level of support in the environment may range from absent or minimal to limited to supportive or highly supportive.(1)(2)(3)(7)(9)(10)(37) [R in Context Link 1, 2, 3, 4, 5, 6, 7, 8]

[S] Participation motivated by a wish to avoid negative consequences rather than accept the need to work toward recovery may require more intense monitoring and follow-up.(8)(37)(38) The patient's level of engagement may be described on a continuum: optimal, positive, limited, minimal, or unengaged. Readiness to change may range from actively and willingly engaged to unable to follow treatment recommendations due to clinical condition.(1)(2)(3)(7)(37) [S in Context Link 1, 2, 3, 4, 5]

[T] Residential care is intended for patients who need around-the-clock behavioral care but do not need the level of physical security and frequency of psychiatric and medical intervention available on an inpatient unit.(1)(2)(3)(4) [T in Context Link 1]

[U] Though state statutes sometimes describe "boot camps" or "wilderness therapy programs" as residential treatment centers, these programs often do not offer the types of services that would be characteristic of a clinical residential treatment center. The American Academy of Child and Adolescent Psychiatry indicates programs that are appropriately classified as clinical residential treatment centers include the use of multidisciplinary teams that include psychologists, psychiatrists, pediatricians, and licensed therapists who consistently are involved in the patient's care. Residential care staffing should include (but not be limited to) a child and adolescent psychiatrist (for adolescent programs, an adult psychiatrist with experience in treatment of this age group may be appropriate), as well as the ability to provide medical care (ill and preventive care) by a qualified primary care provider who is available 24 hours a day, with hospital resources identified as appropriate.(40) [U in Context Link 1]

[V] Composite score does not replace clinical judgment and is meant as a guide to assist in determining needed level of care. Serious risk of harm may independently necessitate placement in residential level of care. Serious functional impairment or major comorbidity may independently necessitate placement at residential level of care unless the patient has minimal stress in the recovery environment and high levels of support.(1)(2)(3) [V in Context Link 1]

[W] Patients appropriate for residential eating disorder treatment may be preoccupied with intrusive or repetitive thoughts for more than 4 hours (up to 6 hours) a day.(6) [W in Context Link 1]

[X] Practice guidelines for the treatment of patients with eating disorders indicate that the decision for treatment of a patient with an eating disorder at a given level of care should be made on an individual basis, taking into account the patient's personal weight status, rate of weight change, risk of developing medical complications, need for and frequency of weight monitoring, comorbidities, motivation, facility-based or home-based supervisory needs, and other psychological, behavioral, and social factors.(6)(12)(16)(23) [X in Context Link 1, 2, 3, 4]

[Y] A practice guideline for the treatment of patients with eating disorders indicates that residential level of care is indicated to provide staff supervision at all meals to limit restrictive eating. Highly structured, around-the-clock care is necessary to support motivation and cooperation with care and help patients develop the skills necessary for recovery.(6) [Y in Context Link 1]

[Z] A practice guideline for the treatment of patients with eating disorders indicates that patients with purging behavior (including use of laxatives and diuretics) who are appropriate for residential level of care are those who can ask for and use support from others or use cognitive and behavioral skills to inhibit purging.(6) [Z in Context Link 1, 2]

[AA] Examples of medical conditions appropriate for residential care include conditions that require active monitoring (either by patient or staff) and have the potential to distract from treatment of the admitting diagnosis, but are not so severe that they require the intensive level of monitoring provided in inpatient treatment.(7) [AA in Context Link 1]

[BB] Goals for care may be diagnostic, therapeutic, or habilitative in nature. Goals should be agreed upon by the patient (or guardian, as appropriate), and incorporate the patient's perspective on current problems, as well as the patient's specific values and preferences. Lack of progress toward goals should trigger a reassessment of the care plan (eg, determination as to accuracy of the admitting diagnosis, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to the modification of the plan as appropriate to promote optimal recovery.(1)(2)(3) [BB in Context Link 1, 2]

[CC] Evaluation of the lack of willingness to participate in treatment (eg, refusal to consent to or participate in treatment) should ascertain whether or not additional efforts to engage the patient in treatment are likely to be successful. It should also include a risk assessment (eg, whether or not the patient would represent a risk to self or others), as well as determine whether or not pursuing involuntary treatment is indicated.(1)(2)(3) [CC in Context Link 1, 2, 3, 4]

[DD] Impairments in motivation to participate in treatment, limitations to engagement in care, and resistance to change are common in patients with mental health and substance use disorders, and may represent a feature of the disease process. A lack of motivation may indicate the need for more intensive services in order to help promote recovery or behavior change. Deficits in motivation or resistance to change should be addressed with therapies designed to enhance motivation to participate in treatment and work toward recovery (eg, motivational enhancement therapy). The level of motivation and goals/strategies designed to address motivation should be included in the care plan, and a lack of progress toward goals should trigger a reassessment of the care plan (eg, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to modification of the plan as appropriate to promote optimal recovery.(1)(2)(3) [DD in Context Link 1, 2, 3, 4]

[EE] A variety of factors may impact the ability of the patient to respond to treatment, including language, cognitive, and intellectual capacity, as well as the presence of any developmental conditions. Assessment of the ability to respond to planned interventions may help identify subgroups of patients more likely to respond to particular treatments, and help optimize (or modify) the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or alternative level of care would be more appropriate to address the patient's needs. Patients who do not initially respond to scheduled therapeutic interventions may respond to modifications of the treatment plans or strategies designed to enhance ability. These include the addition or substitution of a new medication or psychosocial therapy, adjustment of the medication dose, increase in service intensity (some patients require higher levels of service intensity to provide reinforcement to enhance motivation and promote change), or decrease in service intensity (some patients may find higher levels of service intensity so restrictive as to be burdensome and hinder recovery).(1)(2)(3) [EE in Context Link 1, 2, 3, 4]

[FF] The Clinical Institute Withdrawal Assessment tool (CIWA) is used to assess 10 common withdrawal signs and symptoms, including: nausea/vomiting, tremor, anxiety, agitation, paroxysmal sweats; orientation and clouding of sensorium; tactile, auditory, and visual disturbances; and headache. The cumulative score is used to guide treatment management for patients undergoing withdrawal, and ranges from 0 to 67. Withdrawal severity may be categorized by score: mild (less than 10), moderate (10 to 18), severe (19 or greater with the absence of seizure, delirium, or hallucinations), or complicated (19 or greater with the presence of seizure, delirium, or new-onset hallucinations).(49)(50)(51) [FF in Context Link 1]

[GG] The Clinical Opiate Withdrawal Scale (COWS) is used by clinicians to measure 11 commonly observed opiate withdrawal signs or symptoms, including: resting pulse, sweating, restlessness, pupil size, bone or joint aches, runny nose or tearing, gastrointestinal upset, tremor, yawning, anxiety or irritability, and gooseflesh skin. Cumulative scores are described as mild (5 to 12), moderate (13 to 24), moderately severe (25 to 36), and severe (greater than 36). The COWS was developed to guide buprenorphine induction, and can be used to track opiate withdrawal symptoms over time or in response to treatment.(52)(53) [GG in Context Link 1]

[HH] Observation care can be provided in a psychiatric emergency services unit, holding unit, or other facility-based setting; or in a dedicated observation care unit or other hospital-based setting.(59)(60) [HH in Context Link 1]

[II] Participation motivated by a wish to avoid negative consequences rather than accept the need to work toward recovery may require more intense monitoring and follow-up.(8)(37)(38) The patient's level of engagement may be described on a continuum: optimal, positive, limited, minimal, or unengaged. Readiness to change may range from actively and willingly engaged to unable to follow treatment recommendations due to clinical condition.(1)(2)(3)(7)(37) [II in Context Link 1, 2, 3]

[JJ] Short-term (1 to 3 days) crisis stabilization can be accomplished by crisis intervention care that can be done in a facility setting (eg, residential care, short-term psychiatric unit) or by outreach teams.(42)(61)(62) For voluntary patients with acute manifestations of psychiatric disorders who are being considered for inpatient admission because of clinical urgency but are not expected to require physical restraint or around-the-clock nursing care, randomized controlled trials have shown that a residential setting can provide treatment that is no less effective than inpatient care.(61)(63)(64) [JJ in Context Link 1]

[KK] Acute interventions may address multiple factors contributing to a patient's condition including medication needs, identification of needed resources and support, or change in living situation.(67) [KK in Context Link 1]

[LL] Goals for care may be diagnostic, therapeutic, or habilitative in nature. Goals should be agreed upon by the patient (or guardian, as appropriate), and incorporate the patient's perspective on current problems, as well as the patient's specific values and preferences. Lack of progress toward goals should trigger a reassessment of the care plan (eg, determination as to accuracy of the admitting diagnosis, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to the modification of the plan as appropriate to promote optimal recovery.(6)(12) [LL in Context Link 1, 2]

[MM] Evaluation of the lack of willingness to participate in treatment (eg, refusal to consent to or participate in treatment) should ascertain whether or not additional efforts to engage the patient in treatment are likely to be successful. It should also include a risk assessment (eg, whether or not the patient would represent a risk to self or others), as well as determine whether or not pursuing involuntary treatment is indicated.(1)(2)(3)(7) [MM in Context Link 1, 2]

[NN] Impairments in motivation to participate in treatment, limitations to engagement in care, and resistance to change are common in patients with mental health and substance use disorders, and may represent a feature of the disease process. A lack of motivation may indicate the need for more intensive services in order to help promote recovery or behavior change. Deficits in motivation or resistance to change should be addressed with therapies designed to enhance motivation to participate in treatment and work toward recovery (eg, motivational enhancement therapy). The level of motivation and goals/strategies designed to address motivation should be included in the care plan, and a lack of progress toward goals should trigger a reassessment of the care plan (eg, identification of barriers toward progress, incorporation of new problems which may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to modification of the plan as appropriate to promote optimal recovery.(6)(73) [NN in Context Link 1, 2]

[OO] A variety of factors may impact the ability of the patient to respond to treatment, including language, cognitive, and intellectual capacity, as well as the presence of any developmental conditions. Assessment of the ability to respond to planned interventions may help identify subgroups of patients more likely to respond to particular treatments, and help optimize (or modify) the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or alternative level of care would be more appropriate to address the patient's needs. Patients who do not initially respond to scheduled therapeutic interventions may respond to modifications of the treatment plans or strategies designed to enhance ability. These include the addition or substitution of a new medication or psychosocial therapy, adjustment of the medication dose, increase in service intensity (some patients require higher levels of service intensity to provide reinforcement to enhance motivation and promote change), or decrease in service intensity (some patients may find higher levels of service intensity so restrictive as to be burdensome and hinder recovery).(1)(2)(3)(6)(7)(37)(73) [OO in Context Link 1, 2]

[PP] Partial hospital programs provide multidisciplinary behavioral care for 6 to 8 hours per day, 5 to 7 days per week (with patients going home each evening or weekend), and are staffed similarly to the day shift of an inpatient unit. For voluntary patients with acute manifestations of psychiatric disorders who are being considered for inpatient admission because of clinical urgency but are not expected to require around-the-clock behavioral or nursing care, randomized controlled trials have shown that partial hospital programs can provide treatment that is no less effective than inpatient care.(1)(2)(3)(5)(64)(74)(75) [PP in Context Link 1]

[QQ] Patients in partial hospital programs for eating disorders require daily (or near daily) monitoring (including monitoring of medications and overall symptoms, reinforcement of coping skills deficits, substance use, family situation, and observation and safety planning to address potential for self-harm). Family, caregivers, or other supports are able to provide support and structure in addition to care provided by staff.(12)(29) [QQ in Context Link 1]

[RR] Composite score does not replace clinical judgment and is meant as a guide to assist in determining needed level of care. Severe or extreme risk of harm, serious or severe functional impairment, or major or severe comorbidity may independently necessitate a higher level of care.(1)(2)(3)(7) [RR in Context Link 1, 2, 3]

[SS] Partial hospital programs may provide monitoring to prevent refeeding syndrome (in patients with anorexia nervosa) or hourly monitoring of movement or exercise as appropriate to clinical presentation. Family, caregivers, or other supports are able to provide support and structure in addition to care provided by staff.(6)(12) [SS in Context Link 1]

[TT] Patients appropriate for partial hospital care may have intrusive/repetitive thoughts for 3 to 4 hours a day.(6) [TT in Context Link 1]

[UU] A practice guideline for the treatment of patients with eating disorders indicates that patients in partial hospital programs require daily or near daily monitoring (including daily or near daily supervision of meals, monitoring of medication and overall symptoms, reinforcement of coping skills deficits, substance abuse monitoring, family situation monitoring, and observation and safety planning to address potential for self-harm) to address higher levels of impairments. Family, caregivers, or other supports are able to provide support and structure in addition to care provided by staff.(6)(12) [UU in Context Link 1]

[VV] A practice guideline for the treatment of patients with eating disorders indicates that patients with purging behavior (including use of laxatives and diuretics) who are appropriate for partial hospital level of care are able to reduce the number of purging incidents outside of a highly structured treatment setting.(6) [VV in Context Link 1, 2]

[WW] An example of a medical condition appropriate for partial hospital care is asthma that is anticipated to require medication adjustment during the course of treatment.(1)(2)(3) [WW in Context Link 1]

[XX] A practice guideline for partial hospital and intensive outpatient care indicates that admission to a partial hospital program may be preferable to an intensive outpatient program if daily or near daily management or immediate intervention is necessary. Conditions that may require this intensity of monitoring include medication and comprehensive symptom management; observation and safety planning due to danger to self or others; lack of resiliency and need for repeated reinforcement; extreme mood swings, hopelessness, or isolation with inadequate or unavailable community supports; and substance use monitoring. Immediate intervention may be necessary for crisis situations (eg, volatile family situations) or when urgent behavioral activation is required (eg, rapid improvement is necessary to return the individual to vital role functioning).(12) [XX in Context Link 1, 2]

[YY] A practice guideline for partial hospital and intensive outpatient care indicates that admission to a partial hospital or an intensive outpatient program is inappropriate if the patient's needs are primarily social, custodial, recreational, or respite.(12) [YY in Context Link 1, 2]

[ZZ] Goals for care may be diagnostic, therapeutic, or habilitative in nature. Goals should be agreed upon by the patient (or guardian, as appropriate), and incorporate the patient's perspective on current problems, as well as the patient's specific values and preferences. Lack of progress toward goals should trigger a reassessment of

the care plan (eg, determination as to the accuracy of the admitting diagnosis, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to the modification of the plan as appropriate to promote optimal recovery.(1)(2)(3)(12) [ZZ in Context Link 1, 2]

[AAA] A practice guideline for partial hospital and intensive outpatient care indicates that inappropriate candidates for intensive outpatient and partial hospital programs include patients (or parents/guardians, as appropriate) who are uninterested or unable to engage in identifying goals for treatment, contract for behavioral change, or engage in a mutually agreed treatment plan.(12) [AAA in Context Link 1, 2, 3]

[BBB] Partial hospital program staff should verify that support persons are able to conduct any necessary monitoring of the patient's condition, provide instruction regarding the symptoms and behaviors to be monitored, and specify actions to be taken if the patient develops a current plan for suicide or serious harm.(12) [BBB in Context Link 1]

[CCC] A practice guideline for partial hospital and intensive outpatient care indicates that inappropriate candidates for intensive outpatient or partial hospital programs include patients with cognitive dysfunction or a developmental level that precludes the integration of newly learned information, skills, or behavioral change; patients with conditions (eg, social phobia, mania, anxiety, paranoia) assessed likely to be aggravated by care plans that emphasize the role of group treatment; and patients with lack of adherence (either due to unwillingness or lack of ability) to program expectations and personal responsibilities in a manner anticipated to negatively impact group functioning.(12) [CCC in Context Link 1, 2]

[DDD] Intensive outpatient programs (IOPs) typically provide 3 to 4 hours of psychosocial treatment for 1 to 4 days per week (usually 6 to 12 hours of treatment per week), mostly by using a group format, and are appropriate for patients who need a type or frequency of psychosocial treatment that is not currently available in a standard outpatient setting but is available in an intensive outpatient program.(1)(2)(3)(5)(12)(29) [DDD in Context Link 1]

[EEE] A practice guideline for the treatment of patients with eating disorders indicates that an assessment of motivation to recover is indicated in the initial admission assessment to help determine the appropriate level of care to treat the eating disorder.(6) [EEE in Context Link 1]

[FFF] Patients appropriate for IOP may have intrusive/repulsive thoughts, but these do not persist for more than 1 to 2 hours a day in total.(6) [FFF in Context Link 1]

[GGG] A practice guideline for the treatment of patients with eating disorders indicates that patients appropriate for IOP are self-sufficient with meals, although some degree of external structure is required to prevent compulsive exercising.(6)(12) [GGG in Context Link 1]

[HHH] A practice guideline for the treatment of patients with eating disorders indicates that patients with purging behavior (including use of laxatives and diuretics) who are appropriate for IOP are able to reduce the number of purging incidents in an unstructured treatment setting.(6) [HHH in Context Link 1, 2]

[III] Examples of medical conditions appropriate for intensive outpatient care include asthma, hypertension, or diabetes that are being treated on stable medication regimens and not anticipated to require adjustment during the course of treatment.(1)(2)(3)(10) [III in Context Link 1]

[JJJ] Biopsychosocial stressors may impact the level of care necessary to manage a psychiatric or behavioral condition, including the ability of the program to meet comprehensive patient needs, ensure treatment adherence, enhance motivation, or prevent relapse (ie, comorbidities, environmental factors, or other barriers may prevent effective treatment at a less intensive level of care than might otherwise be appropriate to the patient's condition). A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(33)(34)(35)(36) Biopsychosocial assessment factors should be incorporated into care planning, including planned treatment goals, and intensity and duration of interventions. Any identified deficits should be manageable by the program directly or through alternative arrangements.(1)(2)(3)(7) [JJJ in Context Link 1]

[KKK] Outpatient care for eating disorders consists of psychiatric, psychotherapeutic, medical, and dietetic treatment services provided in office or clinic settings for managing acute or subacute manifestations of an eating disorder. Visit frequencies are determined flexibly, on the basis of each patient's needs. For patients who have required inpatient refeeding, the expected duration of outpatient care is 12 months.(6) [KKK in Context Link 1]

[LLL] For patients impaired to a degree to require inpatient care, outpatient or maintenance care follow-up is unlikely to be successful in many treatment delivery systems. Even for well-supported, motivated patients whose symptoms are of short duration, major guidelines suggest that best practice follow-up care includes specialized services that are relatively intensive, multidisciplinary, team-based, and coordinated.(6)(16) [LLL in Context Link 1]

[MMM] A practice guideline for the treatment of patients with eating disorders indicates that an assessment of motivation to recover or engage in care is indicated in the initial admission assessment to assist in determining the appropriate level of care.(6) [MMM in Context Link 1]

[NNN] A practice guideline for the treatment of patients with eating disorders indicates that patients with purging behavior (including use of laxatives and diuretics) who are appropriate for outpatient care are able to reduce the number of purging incidents in an unstructured treatment setting.(6) [NNN in Context Link 1, 2]

[OOO] A practice guideline for the treatment of patients with eating disorders indicates that patients appropriate for outpatient care are able to manage compulsive exercise through self-control.(6) [OOO in Context Link 1]

[PPP] Examples of medical conditions appropriate for outpatient care include asthma on stable treatment regimen or HIV-positive status without symptoms.(1)(2)(3) [PPP in Context Link 1]

[QQQ] Day treatment consists of a community-based mix of psychosocial treatment (including individual, family, and group-based psychotherapy), educational, and recreational activities for patients with behavioral health conditions associated with functional impairment (eg, inability to maintain full-time engagement in work, school, or home environment as appropriate). This level of care typically is monitored by staff whose licensure and credentials (eg, nurses, social workers) is allowed for this role by local statutes and is clinically appropriate to patient needs.(5)(79)(80)(81) Day treatment is designed to address issues that are chronic in nature, rather than acute exacerbations or urgent clinical issues; services tend to overlap with regular school or work schedules, and typically are of longer duration than intensive outpatient or partial hospital programs (eg, an adolescent in day treatment may be enrolled in a program that lasts for the entire school year).(5)(79)(80)(81) [QQQ in Context Link 1]

[RRR] For some patients, day treatment programs provide a period of transition from a more intensive level of care and assist the patient in reintegrating into the community, as enrollment in day treatment following hospitalization has been associated with a decrease in the risk of rehospitalization.(5)(82) While patients for whom day treatment is indicated do not require the intensity of services available in an intensive outpatient or partial hospital program, some day treatment programs provide diagnostic, medical, psychiatric, or other adjunctive treatment modalities, either directly or through arrangements made by the program. These services may be provided over an extended period of time.(79) [RRR in Context Link 1]

[SSS] **Substance-related disorders:** Specialty centers providing outpatient treatment for substance use disorders commonly do so in day treatment centers.(8) Day treatment is appropriate to support abstinence and decrease the risk of relapse, including the promotion of strategies for the management of addiction symptoms (cravings) that pose a risk for relapse but do not require the level of support that would be provided in an intensive outpatient program.(5)(7)(8)(83) Patients appropriate for day treatment may require motivational enhancement and monitoring strategies to address barriers to change (despite at least some readiness for recovery) to help

support engagement in treatment and promote continued progress through the stages of change, but do not require the level of support that would be provided in an intensive outpatient program.(7)(8)(12) Patients with substance use disorders who are appropriate for day treatment are generally resilient and require only minimal support to manage any deficits in the environment.(1)(2)(3)(7) Examples of mild environmental stressors include stress due to normal transition (eg, new school, new job), minor interpersonal loss (eg, loss of peer relationship due to move), transient but significant illness (eg, fractured bone), somewhat inadequate material resources, performance expectations that create discomfort, or potential for exposure to substance use.(1)(2)(3)(7)(8) [SSS in Context Link 1]

[TTT] **Eating disorders:** Patients appropriate for day treatment for eating disorders do not require the level of monitoring that would be provided in an intensive outpatient or partial hospital program (staff supervision of meals is not required), and any supervision of meals that is determined to be clinically appropriate may be provided by family, caregivers, or other supports.(6)(12) [TTT in Context Link 1]

[UUU] Determination of severity may be based on symptom severity alone (including intensity or frequency of symptoms and the extent to which they interfere with functioning), or may be due to other factors (eg, comorbid medical illness, developmental condition, cognitive impairment, substance use disorder, or other factors that contribute to destabilization or decreased ability to cope with the underlying behavioral health disorder). If the comorbidity affects the level of care appropriate to meet the patient's behavioral health needs, the manner in which it impacts the behavioral health condition (and how the comorbidity will be managed at the proposed level of care) should be documented in order to optimize comprehensive patient care.(1)(2)(3)(7)(9) [UUU in Context Link 1]

[VVV] Assessment for comorbid conditions should take place at admission evaluation, and any identified conditions should be manageable by the program (either directly or through alternative arrangements). Comorbidities requiring more active management or that currently distract the patient from therapy or have been associated with challenges in treatment follow-through in the past should prompt consideration of a program designed to provide treatment for both disorders (eg, a dual-diagnosis program).(1)(2)(3)(7)(9)(30) [VVV in Context Link 1]

[WWW] The diagnosis and severity of depressive disorders may be confirmed and documented by the use of standardized rating scales that reliably measure depressive symptoms. For adults, these scales include the Beck Depression Inventory (BDI), the Hamilton Depression Rating Scale (HDRS), the Patient Health Questionnaire-9 (PHQ-9) in the primary care setting, the Montgomery-Asberg Depression Rating Scale (MADRS), and the Geriatric Depression Scale (GDS) for older adults.(84)(85)(86) For children and adolescents, these include the BDI, the Children's Depression Inventory (CDI), and the PHQ-9 modified for adolescents (PHQ-A).(87)(88)(89) As an example, the PHQ-9 and PHQ-A ask the patient to describe the frequency of 9 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day), and depression severity is scored as follows: 0 to 4 none, 5 to 9 mild, 10 to 14 moderate, 15 to 19 moderately severe, and 20 to 27 severe.(90) In addition to screening, rating scales are often administered throughout treatment to measure changes in symptoms. An approximate 50% decrease in scores on the BDI, HDRS, PHQ-9/PHQ-A, or the MADRS has been found to be clinically significant when reassessing an individual for interval improvement.(85)(87)(91) [WWW in Context Link 1]

[XXX] Patients with recent substantial increases in signs and symptoms related to the underlying diagnosis or recent deterioration in function should be considered for a program with a greater level of service intensity (eg, an intensive outpatient or partial hospital program or other higher level of care).(79) [XXX in Context Link 1]

[YYY] Assessment of the likelihood of benefit at the proposed level of care may help identify subgroups of patients more likely to respond to particular treatments and help optimize the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or a different level of service intensity would be more appropriate to address the patient's needs.(1)(2)(3)(6)(7)(8) [YYY in Context Link 1]

[ZZZ] Evidence is inconclusive for animal-assisted interventions for autism spectrum disorder and mental health conditions based on heterogeneity in interventions, methodological weaknesses, small sample sizes, and lack of randomization within studies.(92)(93) [ZZZ in Context Link 1]

Definitions

Acute kidney injury (stage 2)

- Acute kidney injury (stage 2) with significant clinical findings, as indicated by **ALL** of the following(1)(2)(3):
 - Acute[A] kidney injury (stage 2), as indicated by **1 or more** of the following[B]:
 - Within past 7 days, rise in serum creatinine between 2 and 2.9 times its baseline value (increase of 3 times baseline would qualify as stage 3 acute kidney injury)
 - Decreased urine output,[C] as indicated by **ALL** of the following:
 - Assessment of urine output appropriate (eg, adequate volume status, no urinary obstruction)
 - Urine output less than 0.5 mL/kg/hr for 12 hours
 - Significant clinical findings, as indicated by **1 or more** of the following[B]:
 - Hemodynamic instability
 - Worsening clinical status despite observation care (eg, rising creatinine or urine output remains less than 0.5 mL/kg/hr)
 - Altered mental status
 - Cardiac arrhythmias of immediate concern
 - Severe hypertension (SBP greater than 180 mm Hg or DBP greater than 120 mm Hg in adults or greater than the 95th percentile for age, gender, and height plus 30 mm Hg in pediatric patients)(4)(5)
 - Significant respiratory findings (eg, pulmonary edema) that are severe (eg, Hypoxemia) or persist despite observation care
 - Clinically significant electrolyte abnormality that is severe (eg, hyperkalemia with severe ECG findings)[D] or persists despite observation care
 - Clinically significant metabolic abnormality (eg, metabolic acidosis) that is severe or persists despite observation care

References

1. Kidney Disease: Improving Global Outcomes (KDIGO) Acute Kidney Injury Work Group. KDIGO clinical practice guideline for acute kidney injury. Kidney International. Supplement 2012;2(1):1-138. (Reaffirmed 2023 May)
2. Weisbord SD, Palevsky PM. Prevention and management of acute kidney injury. In: Yu AS, Chertow GM, Luyckx VA, Marsden PA, Skorecki K, Taal MW, editors. Brenner and Rector's The Kidney. 11th ed. Philadelphia, PA: Elsevier; 2020:940-977.e15.
3. Palevsky PM, et al. KDOQI US commentary on the 2012 KDIGO clinical practice guideline for acute kidney injury. American Journal of Kidney Diseases 2013;61(5):649-72. DOI: 10.1053/j.ajkd.2013.02.349.
4. Whelton PK, et al. 2017 ACC/AHA/AAPA/ABC/ACPM/AGS/APhA/ASH/ASPC/NMA/PCNA Guideline for the prevention, detection, evaluation, and management of high blood pressure in adults: a report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines. Circulation 2018;138(17):e484-e594. DOI: 10.1161/CIR.0000000000000596. (Reaffirmed 2023 Jan)
5. Flynn JT, et al. Clinical practice guideline for screening and management of high blood pressure in children and adolescents. Pediatrics 2017;140(3):e20171904. DOI: 10.1542/peds.2017-1904. (Reaffirmed 2022 Aug)
6. Mirvis DM, Goldberger AL. Electrocardiography. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine. 12th ed. Elsevier; 2022:141-174.e3.
7. Mount DB. Fluid and electrolyte disturbances. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:338-356.

Footnotes

- A. A specialty society practice guideline defines acute kidney injury as a change in renal function that is known or presumed to have occurred over the previous 7 days.(1)
- B. In determining the stage of acute kidney injury (eg, stage 2 vs stage 3), patients should be staged according to the criteria that give them the highest stage.(1)
- C. The use of urine output as an indicator of acute kidney injury is important, as change in urine output can occur before a noticeable rise in creatinine.(1)(3) It is also suggested that urine output can identify patients with acute kidney injury that may otherwise be missed.(1)(3) However, urine output as a metric is only valid under the appropriate conditions. For example, the patient must not be hypovolemic, there must not be urinary obstruction, and the collection and measurement of urinary output must be complete and accurate.(1)(3) Use of urinary output to define acute kidney injury can be inaccurate in obese patients due to the nonlinear relationship between body weight and urine output. For example, in a 100-kg (220-lb) patient, urine output of 45 mL/hour over 12 hours (adequate output) would qualify as stage 2 acute kidney injury.(1)(3)
- D. ECG findings include peaked T-waves, PR interval greater than 0.20 seconds, QRS interval greater than 0.12 seconds, and arrhythmia.(6)(7)

Acute renal failure (stage 3 acute kidney injury)

- Acute[A] kidney injury (stage 3),[B] as indicated by **1 or more** of the following[C](1)(2)(3):
 - Within past 7 days, rise in serum creatinine to 3 times its baseline value or higher
 - Within past 7 days, rise in serum creatinine to at least 1.5 times its baseline value (increase of 50%) and serum creatinine is greater than 4 mg/dL (354 micromoles/L)
 - In child younger than 18 years, estimated glomerular filtration rate less than 35 mL/min/1.73m² (0.59 mL/sec/1.73m²)  eGFR - Pediatric Calculator and **1 or more** of the following:
 - Within past 7 days, rise in serum creatinine to at least 1.5 times its baseline value (increase of 50%)
 - Within past 48 hours, rise in serum creatinine greater than 0.3 mg/dL (26.5 micromoles/L)
 - Decreased urine output,[D] as indicated by **ALL** of the following:
 - Assessment of urine output appropriate (eg, adequate volume status, no urinary obstruction)
 - Oligoanuria, as indicated by **1 or more** of the following:
 - Urine output less than 0.3 mL/kg/hour for 24 hours
 - Anuria (urine output less than 0.1 mL/kg/hour) for 12 hours
 - Initiation of renal replacement therapy (RRT) required(4)

References

1. Kidney Disease: Improving Global Outcomes (KDIGO) Acute Kidney Injury Work Group. KDIGO clinical practice guideline for acute kidney injury. Kidney International. Supplement 2012;2(1):1-138. (Reaffirmed 2023 May)
2. Weisbord SD, Palevsky PM. Prevention and management of acute kidney injury. In: Yu AS, Chertow GM, Luyckx VA, Marsden PA, Skorecki K, Taal MW, editors. Brenner and Rector's The Kidney. 11th ed. Philadelphia, PA: Elsevier; 2020:940-977.e15.
3. Palevsky PM, et al. KDOQI US commentary on the 2012 KDIGO clinical practice guideline for acute kidney injury. American Journal of Kidney Diseases 2013;61(5):649-72. DOI: 10.1053/j.ajkd.2013.02.349.
4. Gaudry S, Palevsky PM, Dreyfuss D. Extracorporeal kidney-replacement therapy for acute kidney injury. New England Journal of Medicine 2022;386(10):964-975. DOI: 10.1056/NEJMra2104090.

Footnotes

- A. A specialty society practice guideline defines acute kidney injury as a change in renal function that is known or presumed to have occurred over the previous 7 days.(1)
- B. For purposes of these criteria, the term acute renal failure can be viewed as stage 3 acute kidney injury in the KDIGO classification system.(1) Of note, in the ICD-10 diagnostic coding system (main coding scheme used in labeling diagnoses), the relevant codes covering the clinical entity of significant acute kidney injury use the term acute kidney failure (eg, code N17.9).
- C. In determining the stage of acute kidney injury (eg, stage 2 vs stage 3), patients should be staged according to the criteria that give them the highest stage.(1)
- D. The use of urine output as an indicator of acute kidney injury is important, as change in urine output can occur before a noticeable rise in creatinine.(1)(3) It is also suggested that urine output can identify patients with acute kidney injury that may otherwise be missed.(1)(3) However, urine output as a metric is only valid under the appropriate conditions. For example, the patient must not be hypovolemic, there must not be urinary obstruction, and the collection and measurement of urinary output must be complete and accurate.(1)(3) Use of urinary output to define acute kidney injury can be inaccurate in obese patients due to the nonlinear relationship between body weight and urine output. For example, in a 100-kg (220-lb) patient, urine output of 45 mL/hour over 12 hours (adequate output) would qualify as stage 2 acute kidney injury.(1)(3)

Altered mental status

- Altered mental status (ie, different from baseline), as indicated by **1 or more** of the following(1)(2)(3)(4):
 - Confusional state (eg, disorientation, difficulty following commands, deficit in attention)
 - Lethargy (eg, awake or arousable, but with drowsiness; reduced awareness of self and environment)
 - Obtundation (ie, arousable only with strong stimuli, lessened interest in environment, slowed responses to stimulation)
 - Stupor (ie, may be arousable but patient does not return to normal baseline level of awareness)
 - Coma (ie, not arousable)

References

1. Maher PJ. Confusion. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:126-131.e1.
2. Lei C, Smith C. Depressed consciousness and coma. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:117-125.e1.
3. Abraham G, Maher PJ. Delirium and dementia. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1269-1280.e2.
4. Schor NF. Neurologic evaluation. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:3053-3063.e1.

Anxiety (adult)

- Multiple instruments are available to assess anxiety severity. The General Anxiety Disorder-7 (GAD-7) asks the patient to describe the frequency of 7 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day). A score of 5 to 9 suggests mild anxiety; 10 to 14, moderate anxiety; and 15 or greater, severe anxiety.(1) Examples of anxiety symptoms include(2):
 - Feeling nervous, anxious, or on edge
 - Not being able to stop or control worrying
 - Worrying too much about different things
 - Trouble relaxing
 - Being so restless that it is hard to sit still
 - Becoming easily annoyed or irritable
 - Feeling afraid, as if something awful might happen

References

- Spitzer RL, Kroenke K, Williams JB, Lowe B. A brief measure for assessing generalized anxiety disorder: the GAD-7. Archives of Internal Medicine 2006;166(10):1092-7. DOI: 10.1001/archinte.166.10.1092.
- Anxiety disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:215-261.

Anxiety (child or adolescent)

- Multiple instruments are available to assess anxiety severity, including the Multidimensional Anxiety Scale for Children-2nd Edition (MASC 2), the Screen for Child Anxiety-Related Emotional Disorders (SCARED), and the Spence Children's Anxiety Scale (SCAS).(1)(2)(3)(4)(5) Examples of anxiety symptoms include(6):
 - Nervousness
 - Feeling on edge
 - Inability to stop worrying or excessive worrying
 - Being told by others that one worries too much
 - Trouble relaxing
 - Inability to sit still due to restlessness
 - Easily irritable or annoyed
 - Feeling afraid that something awful might happen
 - Feeling frightened for no reason
 - Feeling afraid to be alone in the house
 - Feeling scared to go to school

References

- Wehry AM, Beesdo-Baum K, Hennelly MM, Connolly SD, Strawn JR. Assessment and treatment of anxiety disorders in children and adolescents. Current Psychiatry Reports 2015;17(7):52. DOI: 10.1007/s11920-015-0591-z.
- March JS, Parker JD, Sullivan K, Stallings P, Conners CK. The Multidimensional Anxiety Scale for Children (MASC): factor structure, reliability, and validity. Journal of the American Academy of Child and Adolescent Psychiatry 1997;36(4):554-65. DOI: 10.1097/00004583-199704000-00019.
- Fraccaro RL, Stelnicki AM, Nordstokke DW. Test review: Multidimensional Anxiety Scale for Children by J. S. March. Canadian Journal of School Psychology 2015;30(1):70-77. DOI: 10.1177/0829573514542924.
- Birmaher B, Brent DA, Chiappetta L, Bridge J, Monga S, Baugher M. Psychometric properties of the Screen for Child Anxiety Related Emotional Disorders (SCARED): a replication study. Journal of the American Academy of Child and Adolescent Psychiatry 1999;38(10):1230-6. DOI: 10.1097/00004583-199910000-00011.
- Spence SH. A measure of anxiety symptoms among children. Behaviour Research and Therapy 1998;36(5):545-66.
- Anxiety disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:215-261.

Cardiac arrhythmias of immediate concern

- Cardiac arrhythmias or findings of immediate concern, as indicated by **1 or more** of the following(1)(2)(3):
 - Heart rhythms that are inherently dangerous or unstable, as indicated by **1 or more** of the following(4)(5)(6):
 - Resuscitated ventricular fibrillation or cardiac arrest
 - Ventricular escape rhythm
 - Sustained ventricular tachycardia (30 seconds or more of ventricular rhythm at greater than 100 beats per minute)
 - Nonsustained ventricular tachycardia and **1 or more** of the following:
 - Suspected cardiac ischemia as cause or consequence of ventricular tachycardia
 - Acute myocarditis
 - Unstable cardiac conduction defects, as indicated by **1 or more** of the following(6)(7)(8):
 - Type II second-degree atrioventricular block
 - Third-degree atrioventricular block
 - New-onset left bundle branch block with suspected myocardial ischemia
 - Any heart rhythm and **1 or more** of the following(4)(5)(9)(10)(11):
 - Continuous long-term ECG monitoring needed (eg, initiation of drug requiring monitoring beyond observation care)
 - Patient has automatic implanted cardioverter-defibrillator that is repeatedly firing, malfunctioning, or in need of immediate adjustment of settings beyond scope of observation care.
 - Heart rhythms of concern due to **1 or more** of the following:
 - Hypotension
 - Respiratory distress
 - Association with other significant symptoms (eg, syncope)(9)(10)(12)

References

- Curtis AB, Tomaselli GF. Approach to the patient with cardiac arrhythmias. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine. 12th ed. Elsevier; 2022:1145-1162.e7.
- Dalal AS, Van Hare GF. Disturbances of rate and rhythm of the heart. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:2436-2447.e1.

3. Joglar JA, et al. 2023 HRS Expert Consensus Statement on the management of arrhythmias during pregnancy. *Heart Rhythm* 2023;20(10):Online. DOI: 10.1016/j.hrthm.2023.05.017.
4. El-Chami M, Fernando L. Ventricular arrhythmias. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. *Principles and Practice of Hospital Medicine*. 2nd ed. New York, NY: McGraw-Hill Education; 2017:1003-1014.
5. Al-Khatib SM, et al. 2017 AHA/ACC/HRS guideline for management of patients with ventricular arrhythmias and the prevention of sudden cardiac death: a report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines and the Heart Rhythm Society. *Circulation* 2018;138(13):e272-e391. DOI: 10.1161/CIR.0000000000000549. (Reaffirmed 2023 Jul)
6. Epstein AE, et al. 2012 ACCF/AHA/HRS focused update incorporated into the ACCF/AHA/HRS 2008 guidelines for device-based therapy of cardiac rhythm abnormalities: a report of the American College of Cardiology Foundation/American Heart Association Task Force on Practice Guidelines and the Heart Rhythm Society. *Circulation* 2013;127(3):e283-e352. DOI: 10.1161/CIR.0b013e318276ce9b. (Reaffirmed 2022 Jun)
7. Miller JM, Ellenbogen KA. Therapy for cardiac arrhythmias. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. *Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine*. 12th ed. Elsevier; 2022:1208-1244.
8. Park DS, Fishman GI. The cardiac conduction system. *Circulation* 2011;123(8):904-15. DOI: 10.1161/CIRCULATIONAHA.110.942284.
9. Joglar JA, et al. 2023 ACC/AHA/ACCP/HRS Guideline for the diagnosis and management of atrial fibrillation: a report of the American College of Cardiology/American Heart Association joint committee on clinical practice guidelines. *Journal of the American College of Cardiology* 2023 Nov;online. DOI: 10.1016/j.jacc.2023.08.017. (Reaffirmed 2023 Nov 30)
10. Chun EB, McGorisk GM. Supraventricular tachyarrhythmias. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. *Principles and Practice of Hospital Medicine*. 2nd ed. New York, NY: McGraw-Hill Education; 2017:980-995.
11. Hoskins MH, De Lurgio DB. Pacemakers, defibrillators, and cardiac resynchronization devices in hospital medicine. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. *Principles and Practice of Hospital Medicine*. 2nd ed. New York, NY: McGraw-Hill Education; 2017:1025-34.
12. Westerman S, Manogue M, Hoskins MH. Bradycardia. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. *Principles and Practice of Hospital Medicine*. 2nd ed. New York, NY: McGraw-Hill Education; 2017:996-1002.

Command auditory hallucinations

- An auditory hallucination is a "false perception of sound." This is the most common type of hallucination in psychiatric disorders and typically consists of voices but also may consist of any type of sound. A command auditory hallucination is a false perception of a voice giving orders to the patient that they may feel compelled to obey. Likely adherence to a command auditory hallucination is indicated by **1 or more** of the following(1)(2)(3)(4):
 - Perceived familiar or benevolent source
 - No belief by patient in their ability to control the voices
 - History of frequent response to such hallucinations with unquestioning obedience

References

1. Lewis SF, Escalona R, Keith SJ. Phenomenology of schizophrenia. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1406-1425.
2. Cohen CI, Izediuno I, Yadack AM, Ghosh B, Garrett M. Characteristics of auditory hallucinations and associated factors in older adults with Schizophrenia. *American Journal of Geriatric Psychiatry* 2014;22(5):442-9. DOI: 10.1016/j.jagp.2013.07.001.
3. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.
4. Clementz BA, Keshavan MS, Pearlson GD, Sweeney JA, Tamminga CA. Phenotypes of psychosis. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1512-1519.

Current plan

- Determining the existence of current plan for suicide, homicide, or serious harm to self or another involves clinical judgment regarding multiple factors. Higher risk may be indicated by a combination of factors including explicit or implicit evidence of **1 or more** of the following(1)(2):
 - Identification and/or availability of severe or lethal method (eg, using a gun)
 - Preparatory acts (eg, writing suicide note)
 - Intention that act will have severe or fatal outcome evidenced by planning where and when action will be taken (eg, act unlikely to be discovered and likely to be fatal)

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Danger to others for adult

- Danger to others for adult is present due to **1 or more** of the following(1)(2)(3):
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another
 - Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following(A):
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (adult) or unreliability
 - Indication or report of significant physical or sexual aggression with Impaired impulse control, judgment, or insight that significantly endangers another
 - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

3. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Danger to others for child or adolescent

- Danger to others for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another
 - Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability
 - Indication or report of significant physical or sexual aggression with Impaired impulse control, judgment, or insight that significantly endangers another
 - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. Newcorn JH, Kufert Y, Ivanov I, Chacko A. Aggression and violence. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:581-604.
5. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. Pediatrics 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul)
6. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]
7. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(7)

Danger to self for adult

- Danger to self for adult is present due to **1 or more** of the following(1)(2)(3):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (adult) or unreliability
 - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
3. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Danger to self for child or adolescent

- Danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self(4)
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care as indicated by **1 or more** of the following(A):
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability
 - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.
5. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. Pediatrics 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul)
6. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(4)

Dehydration that is severe or persistent

- Dehydration that is severe or persistent, as indicated by **1 or more** of the following(1)(2)(3)(4):
 - Clinical findings of severe dehydration, as indicated by **1 or more** of the following:
 - Hemodynamic instability
 - Acute renal failure (stage 3 acute kidney injury)
 - Acute kidney injury (stage 2)
 - Serum sodium greater than 150 mEq/L (mmol/L)
 - Dehydration that is persistent, as indicated by **ALL** of the following:
 - Oral rehydration therapy not tolerated or insufficient to adequately correct dehydration
 - Appropriate intravenous treatment (eg, fluids) does not readily correct dehydration.

References

1. Padilipsky P, White W. Pediatric infectious diarrheal disease and dehydration. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:2151-2168.e3.
2. Mount DB. Fluid and electrolyte disturbances. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:338-356.
3. Greenbaum LA. Deficit therapy. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:429-432.e1.
4. Greenbaum LA. Maintenance and replacement therapy. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:425-428.e1.

Delusions (adult)

- Delusions are fixed beliefs that are not amenable to change in light of conflicting evidence. Common themes include persecutory, grandiose, erotomanic, nihilistic, and somatic. Delusions are considered bizarre if they are clearly implausible, not understandable to peers of the same culture, and do not derive from ordinary life experiences. Thought withdrawal, thought insertion, and delusions of control are examples of bizarre delusions.(1) Delusion severity may be described as(2)(3)(4):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

References

1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.

- Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
- Kay SR, Fiszbein A, Opler LA. The positive and negative syndrome scale (PANSS) for schizophrenia. Schizophrenia Bulletin 1987;13(2):261-276.
- The PANSS Institute. [Internet] The PANSS Institute. Accessed at: <http://www.panss.org/>. Updated 2023 [accessed 2023 Sep 12]

Delusions (child or adolescent)

- Delusions are fixed beliefs that are not amenable to change in light of conflicting evidence. Common themes include persecutory, grandiose, erotomanic, nihilistic, and somatic. Delusions are considered bizarre if they are clearly implausible, not understandable to peers of the same culture, and do not derive from ordinary life experiences. Thought withdrawal, thought insertion, and delusions of control are examples of bizarre delusions. Instruments are available for assessing severity of symptoms in young people, including the instrument for positive and negative symptoms in severely disturbed children and adolescents (Kiddie-PANSS). Delusion severity may be described as (1)(2):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

- Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
- Fields JH, et al. Assessing positive and negative symptoms in children and adolescents. American Journal of Psychiatry 1994;151(2):249-53.

Depressive symptoms (adult)

- Multiple instruments are available to assess depression severity and may be used to measure changes in depressive symptoms throughout treatment. For example, the Patient Health Questionnaire-9 (PHQ-9) for adults, used in the primary care setting, assesses how frequently the patient experienced the following symptoms over the past 2 weeks[A](1)(2)(3)(4)(5):
 - Interest in activities
 - Depressed mood/hopelessness
 - Disturbed sleep
 - Decreased energy
 - Change in appetite
 - Self-loathing
 - Difficulty concentrating
 - Psychomotor retardation
 - Suicidal thoughts

References

- Bukumiric Z, et al. Meta-analysis of the changes in correlations between depression instruments used in longitudinal studies. Journal of Affective Disorders 2016;190:733-743. DOI: 10.1016/j.jad.2015.10.054.
- Kroenke K, Spitzer RL, Williams JB. The PHQ-9: validity of a brief depression severity measure. Journal of General Internal Medicine 2001;16(9):606-13.
- Maurer DM, Raymond TJ, Davis BN. Depression: screening and diagnosis. American Family Physician 2018;98(8):508-515.
- Spitzer RL, Williams JB, Kroenke K. Patient Health Questionnaire-9 (PHQ-9). [Internet] Pfizer, Inc. Accessed at: <https://www.phqscreeners.com/>. [accessed 2023 Aug 10]
- Riedel M, et al. Response and remission criteria in major depression--a validation of current practice. Journal of Psychiatric Research 2010;44(15):1063-8. DOI: 10.1016/j.jpsychires.2010.03.006.

Footnotes

- A. The diagnosis and severity of depressive disorders may be confirmed and documented by the use of standardized rating scales that reliably measure depressive symptoms in adults, including the Beck Depression Inventory (BDI), the Hamilton Depression Rating Scale (HDRS), the Patient Health Questionnaire-9 (PHQ-9) in the primary care setting, the Montgomery-Asberg Depression Rating Scale (MADRS), and the Geriatric Depression Scale (GDS) for older adults.(1)(2)(3) As an example, the PHQ-9 asks the patient to describe the frequency of 9 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day), and depression severity is scored as follows: 0 to 4 none, 5 to 9 mild, 10 to 14 moderate, 15 to 19 moderately severe, and 20 to 27 severe.(4) In addition to screening, rating scales are often administered throughout treatment to measure changes in symptoms. An approximate 50% decrease in scores on the BDI, HDRS, PHQ-9, and/or MADRS has been found to be clinically significant when reassessing an individual for interval improvement.(2) (5)

Depressive symptoms (child or adolescent)

- Multiple instruments are available to assess depression severity and may be used to measure changes in depressive symptoms throughout treatment. For example, the Patient Health Questionnaire-9 (PHQ-9) modified for adolescents (PHQ-A), used in the primary care setting, assesses how frequently the patient experienced symptoms over the past 2 weeks. Of note, the PHQ-A has been validated for use with adolescents, but has not been validated for use with children younger than 11 years of age. Symptoms assessed include[A](1)(2)(3)(4)(5)(6)(7)(8):

- Interest in activities
- Depressed mood/hopelessness
- Disturbed sleep
- Decreased energy
- Change in appetite
- Self-loathing
- Difficulty concentrating
- Psychomotor retardation
- Suicidal thoughts

References

1. US Preventive Services Task Force, et al. Screening for depression and suicide risk in children and adolescents: US Preventive Services Task Force recommendation statement. *Journal of the American Medical Association* 2022;328(15):1534-1542. DOI: 10.1001/jama.2022.16946. (Reaffirmed 2022 Oct)
2. Richardson LP, et al. Evaluation of the Patient Health Questionnaire-9 Item for detecting major depression among adolescents. *Pediatrics* 2010;126(6):1117-23. DOI: 10.1542/peds.2010-0852.
3. Allgaier AK, Pietsch K, Fruhe B, Sigl-Glockner J, Schulte-Korne G. Screening for depression in adolescents: validity of the patient health questionnaire in pediatric care. *Depression and Anxiety* 2012;29(10):906-13. DOI: 10.1002/da.21971.
4. Spitzer RL, Williams JB, Kroenke K. Patient Health Questionnaire-9 (PHQ-9). [Internet] Pfizer, Inc. Accessed at: <https://www.phqscreeners.com/>. [accessed 2023 Aug 10]
5. Riedel M, et al. Response and remission criteria in major depression—a validation of current practice. *Journal of Psychiatric Research* 2010;44(15):1063-8. DOI: 10.1016/j.jpsychires.2010.03.006.
6. Kroenke K, Spitzer RL, Williams JB. The PHQ-9: validity of a brief depression severity measure. *Journal of General Internal Medicine* 2001;16(9):606-13.
7. Lewandowski RE, et al. Screening for and diagnosis of depression among adolescents in a large health maintenance organization. *Psychiatric Services* 2016;67(6):636-41. DOI: 10.1176/appi.ps.201400465.
8. Richardson LP, et al. Collaborative care for adolescents with depression in primary care: a randomized clinical trial. *Journal of the American Medical Association* 2014;312(8):809-16. DOI: 10.1001/jama.2014.9259.

Footnotes

- A. The diagnosis and severity of depressive disorders may be confirmed and documented by the use of standardized rating scales that reliably measure depressive symptoms in children and adolescents, including the Beck Depression Inventory (BDI), the Children's Depression Inventory (CDI), and the Patient Health Questionnaire-9 (PHQ-9) modified for adolescents (PHQ-A) in the primary care setting.(1)(2)(3) As an example, the PHQ-9 asks the patient to describe the frequency of 9 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day), and depression severity is scored as follows: 0 to 4 none, 5 to 9 mild, 10 to 14 moderate, 15 to 19 moderately severe, and 20 to 27 severe.(4) In addition to screening, rating scales are often administered throughout treatment to measure changes in symptoms. An approximate 50% decrease in scores on the BDI and PHQ-A has been found to be clinically significant when reassessing an individual for interval improvement.(1)(5)(6)

Hallucinations (adult)

- Hallucinations are perceptions that occur without an external stimulus (eg, hearing, seeing, tasting, smelling, or feeling things that are not present). They may occur in any sensory modality, but auditory hallucinations are the most common in schizophrenia and related disorders.(1) Hallucination severity may be described as(2)(3) (4):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

References

1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders*. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
2. Assessment measures. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders*. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. Kay SR, Fiszbein A, Opler LA. The positive and negative syndrome scale (PANSS) for schizophrenia. *Schizophrenia Bulletin* 1987;13(2):261-276.
4. The PANSS Institute. [Internet] The PANSS Institute. Accessed at: <http://www.panss.org/>. Updated 2023 [accessed 2023 Sep 12]

Hallucinations (child or adolescent)

- Hallucinations are perceptions that occur without an external stimulus (eg, hearing, seeing, tasting, smelling, or feeling things that are not present). They may occur in any sensory modality, but auditory hallucinations are the most common in schizophrenia and related disorders. Instruments are available for assessing the severity of symptoms in young people, including the instrument for positive and negative symptoms in severely disturbed children and adolescents (Kiddie-PANSS).(1)(2)

References

1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders*. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
2. Fields JH, et al. Assessing positive and negative symptoms in children and adolescents. *American Journal of Psychiatry* 1994;151(2):249-53.

Harm

- Harm to self or another is considered serious if it has a substantial likelihood of causing death, disability, or major disfigurement.(1)(2)(3)(4)(5)

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
3. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
4. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
5. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Hemodynamic instability

- Hemodynamic instability, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Vital sign abnormality not readily corrected by appropriate treatment, as indicated by **1 or more** of the following[A]:
 - Tachycardia that persists despite appropriate treatment (eg, volume repletion, treatment of pain, treatment of underlying cause)
 - Hypotension that persists despite appropriate treatment (eg, volume repletion, treatment of underlying cause)
 - Orthostatic hypotension that persists despite appropriate treatment (eg, volume repletion)
 - Hypotension that is severe, as indicated by **ALL** of the following:
 - Hypotension
 - Severe hypotension, as indicated by **1 or more** of the following:
 - Lactate of 2.0 mmol/L (18 mg/dL) or more secondary to hypotension (ie, hypoperfusion)[B](3)(6)
 - Metabolic acidosis (arterial or venous[C] pH less than 7.35) not otherwise explained
 - Mean arterial pressure[A][D] less than 65 mm Hg
 - IV inotropic or vasopressor medication required to maintain adequate blood pressure or perfusion

References

1. Puskari MA, Jones AE. Shock. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:34-41.e1.
2. Lewis J, Patel B. Shock. In: Gershel JC, Rauch DA, editors. Caring for the Hospitalized Child: A Handbook of Inpatient Pediatrics. 3rd ed. Elk Grove Village, IL: American Academy of Pediatrics; 2023:107-114.
3. Singer M, et al. The Third International Consensus definitions for sepsis and septic shock (Sepsis-3). Journal of the American Medical Association 2016;315(8):801-810. DOI: 10.1001/jama.2016.0287.
4. Turner DA, Cheifetz IM. Shock. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:572-583.e1.
5. Nyhan A. Cardiology. In: Kleinman K, McDaniel L, Molloy M, editors. The Harriet Lane Handbook: A Manual for Pediatric House Officers. 22nd ed. 202: Elsevier; 2021:145-188.e4.
6. Naidu SS, et al. SCAI SHOCK stage classification expert consensus update: a review and incorporation of validation studies: this statement was endorsed by the American College of Cardiology (ACC), American College of Emergency Physicians (ACEP), American Heart Association (AHA), European Society of Cardiology (ESC) Association for Acute Cardiovascular Care (ACVC), International Society for Heart and Lung Transplantation (ISHLT), Society of Critical Care Medicine (SCCM), and Society of Thoracic Surgeons (STS) in December 2021. Journal of the American College of Cardiology 2022;Online. DOI: 10.1016/j.jacc.2022.01.018.
7. Wardi G, Brice J, Correia M, Liu D, Self M, Tainter C. Demystifying lactate in the emergency department. Annals of Emergency Medicine 2020;75(2):287-298. DOI: 10.1016/j.annemergmed.2019.06.027.
8. Levitt DG, Levitt JE, Levitt MD. Quantitative assessment of blood lactate in shock: measure of hypoxia or beneficial energy source. BioMed Research International 2020;2020:2608318. DOI: 10.1155/2020/2608318.
9. Bakker J, Postelnicu R, Mukherjee V. Lactate: where are we now? Critical Care Clinics 2020;36(1):115-124. DOI: 10.1016/j.ccc.2019.08.009.
10. Kraut JA, Madias NE. Lactic acidosis. New England Journal of Medicine 2014;371(24):2309-2319. DOI: 10.1056/NEJMr1309483.
11. Olivieri L, Chasm R. Diabetic ketoacidosis in the pediatric emergency department. Emergency Medicine Clinics of North America 2013;31(3):755-773. DOI: 10.1016/j.emc.2013.05.004.
12. Maloney GE, Glauser JM. Diabetes mellitus and disorders of glucose homeostasis. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1543-1558.e2.
13. Arnold TD, Miller M, van Wessel KP, Evans JA, Balogh ZJ. Base deficit from the first peripheral venous sample: a surrogate for arterial base deficit in the trauma bay. Journal of Trauma 2011;71(4):793-7; discussion 797. DOI: 10.1097/TA.0b013e31822ad694.
14. Malinoski DJ, Todd SR, Slone S, Mullins RJ, Schreiber MA. Correlation of central venous and arterial blood gas measurements in mechanically ventilated trauma patients. Archives of Surgery 2005;140(11):1122-5. DOI: 10.1001/archsurg.140.11.1122.
15. Zakrisson T, McFarlan A, Wu YY, Keshet I, Nathens A. Venous and arterial base deficits: do these agree in occult shock and in the elderly? A Bland-Altman analysis. Journal of Trauma and Acute Care Surgery 2013;74(3):936-9. DOI: 10.1097/TA.0b013e318282747a.

Footnotes

- A. Criteria based upon clinician-acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. There are numerous causes of an elevated lactate level. The most common are cardiogenic or hypovolemic shock, severe heart failure, severe trauma, or sepsis. However, there are also other etiologies to consider such as vigorous exercise, seizures, liver disease, or medication use (eg, metformin, beta2-agonists); therefore, interpretation of elevated lactate levels requires consideration of the clinical context (eg, well-appearing post exercise vs hypotensive). The severity and persistence of the elevation can sometimes be helpful in this differentiation. In most instances of lactic acidosis, blood pH is less than 7.35, with a serum bicarbonate level 20 mEq/L (mmol/L) or lower. However, a coexisting respiratory or metabolic alkalosis can mask these findings.(6)(7)(8)(9)(10)
- C. Analyses have concluded that venous and arterial pH measurements are highly correlated, with small differences between them that are usually not clinically significant.(11)(12)(13)(14)(15) If a mixed acid-base disorder is suspected, an arterial blood gas is indicated.(12)

- D. The mean arterial pressure takes into account both systolic and diastolic blood pressure readings and is calculated as mean arterial pressure (MAP) equals $1/3 \text{ SBP} + 2/3 \text{ DBP}$.(1)(4) A calculator is available at <https://www.mdcalc.com/mean-arterial-pressure-map>.

Hyperactivity or inattention (adult)

- Hyperactivity refers to increased movement, impulsive actions, a decreased attention span, and easy distractibility. Inattention refers to easy distractibility and deficits in the ability to focus or stay on task. Determining severity includes the number of hyperactive or inattentive symptoms present, the frequency and duration of symptom presentation, and the impact that symptoms have on function and development. Levels of severity may be described as(1)(2):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

References

- Neurodevelopmental disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:35-99.
- Kessler RC, et al. The World Health Organization Adult ADHD Self-Report Scale (ASRS): a short screening scale for use in the general population. Psychological Medicine 2005;35(2):245-56.

Hyperactivity or inattention (child or adolescent)

- Hyperactivity refers to increased movement, impulsive actions, a decreased attention span, and easy distractibility. Inattention refers to easy distractibility and deficits in the ability to focus or stay on task. Determining severity includes the number of hyperactive or inattentive symptoms present, the frequency and duration of symptom presentation, and the impact that symptoms have on function and development. Levels of severity may be described as(1):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

- Neurodevelopmental disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:35-99.

Hypotension

- Hypotension, as indicated by **ALL** of the following(1)(2)(3)(4):
 - Not patient baseline (eg, healthy adult with low SBP) or intentional therapeutic goal (eg, low SBP as treatment goal in heart failure)
 - Low blood pressure, as indicated by **1 or more** of the following:
 - SBP less than 90 mm Hg in adult or child 10 years or older[A]
 - Mean arterial pressure[A][B] less than 70 mm Hg in adult or child 10 years or older
 - Decrease in baseline mean arterial pressure of 25% or more,[A][B] with significant signs or symptoms due to lower blood pressure (eg, near syncope, syncope, chest pain)
 - SBP less than sum of 70 mm Hg plus twice patient's age in years in child 1 to 9 years of age[A]
 - SBP less than 70 mm Hg in infant 1 to 11 months of age[A]

References

- Jones D, Di Francesco L. Hypotension. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:657-64.
- Massaro AF. Approach to the patient with shock. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:2235-2241.
- Horeczko T. Pediatric cardiac disorders. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:2109-2131.e1.
- Singh S, Holmes JF. Pediatric trauma. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:2052-2066.e3.

Footnotes

- A. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. The mean arterial pressure takes into account both systolic and diastolic blood pressure readings and is calculated as Mean Arterial Pressure (MAP) = $1/3 \text{ SBP} + 2/3 \text{ DBP}$. A calculator is available at <https://www.mdcalc.com/mean-arterial-pressure-map>.

Hypoxemia

- Hypoxemia, as indicated by **1 or more** of the following(1):
 - Patient without baseline need for supplemental oxygen with oxygen saturation (SaO₂) of less than 90% or arterial blood gas partial pressure of oxygen (PO₂) of less than 60 mm Hg (8.0 kPa) on room air[A][B]
 - Patient without baseline need for supplemental oxygen who now requires supplemental oxygen to keep SaO₂ greater than 89% or PO₂ greater than 59 mm Hg (7.9 kPa)[A]
 - Patient with baseline need for supplemental oxygen who now requires increased supplemental oxygen to maintain oxygenation at baseline or acceptable level

References

- Dezube R, Lechtzin N. Hypoxia. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:669-675.
- Jamali H, et al. Racial disparity in oxygen saturation measurements by pulse oximetry: evidence and implications. Annals of the American Thoracic Society 2022;19(12):1951-1964. DOI: 10.1513/AnnalsATS.202203-270CME.
- Rojas-Camayo J, et al. Reference values for oxygen saturation from sea level to the highest human habitation in the Andes in acclimatised persons. Thorax 2018;73(8):776-778. DOI: 10.1136/thoraxjnl-2017-210598.

Footnotes

- A. Pulse oximetry (SpO₂) may underestimate arterial saturation (SaO₂), especially in people with dark skin pigmentation, thick skin, cold body temperature, or who are wearing colored nail polish, and thus may not accurately detect hypoxemia.(2) Where appropriate, pulse oximetry should be measured after warming fingers or removing nail polish. Pulse oximetry results should be assessed within the context of the patient's clinical presentation, and performance of an arterial blood gas considered for a more accurate assessment of oxygenation if indicated.(2) Specific target values may require adjustment when care is delivered at high altitude.(1)(3)
- B. Criteria based upon clinician-acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.

Imminent danger to others for adult

- Imminent danger to others for adult is present due to **1 or more** of the following(1)(2)(3):
 - Imminent risk for recurrence of attempt to seriously Harm another is present, as indicated by **ALL** of the following:
 - There has been a recent attempt to seriously Harm another.
 - There has not been Sufficient relief of factors that precipitated the attempt.
 - Current plan for homicide or serious Harm to another is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another are present.
 - Patient has persistent thoughts of homicide or violent acts that likely could result in serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (adult), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Indication or report of repeated behavior, including physical or sexual aggression that is clearly injurious to others with history, plan or intent, and extremely Impaired impulse control, judgment, or insight

References

- American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
- APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
- The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Imminent danger to others for child or adolescent

- Imminent danger to others for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Imminent risk for recurrence of attempt to seriously Harm another is present, as indicated by **ALL** of the following:
 - There has been a recent attempt to seriously Harm another.

- There has not been Sufficient relief of factors that precipitated the attempt.
- Current plan for homicide or serious Harm to another is present.
- Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another are present.
- Patient has persistent thoughts of homicide or violent acts that likely could result in serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Severe conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Indication or report of repeated behavior, including physical or sexual aggression that is clearly injurious to others with history, plan or intent, and extremely Impaired impulse control, judgment, or insight

References

1. Baron DA, Cobb RT, Juarez GM. Other psychiatric emergencies. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2622-2637.
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
3. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
4. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
5. Newcorn JH, Kufert Y, Ivanov I, Chacko A. Aggression and violence. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:581-604.
6. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]
7. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(7)

Imminent danger to self for adult

- Imminent danger to self for adult is present due to **1 or more** of the following(1)(2)(3):
 - Imminent risk for recurrence of Suicide attempt or act of serious Harm to self is present, as indicated by **ALL** of the following:
 - There has been a recent Suicide attempt or deliberate act of serious Harm to self.
 - There has not been Sufficient relief of the factors that precipitated attempt or act.
 - Current plan for suicide or serious Harm to self is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for suicide or serious Harm to self are present.
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (adult), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Frantic hopelessness; fatalistic conviction that life will not improve, along with oppressive sense of entrapment and doom

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
3. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Imminent danger to self for child or adolescent

- Imminent danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Imminent risk for recurrence of Suicide attempt or act of serious Harm to self is present, as indicated by **ALL** of the following:
 - There has been a recent Suicide attempt or deliberate act of serious Harm to self.
 - There has not been Sufficient relief of the factors that precipitated attempt or act.
 - Current plan for suicide or serious Harm to self is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for suicide or serious Harm to self are present.

- Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following^[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Severe conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Frantic hopelessness; fatalistic conviction that life will not improve, along with oppressive sense of entrapment and doom

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.
5. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]
6. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. Pediatrics 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul)

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.⁽⁴⁾

Impaired impulse control, judgment, or insight

- Impaired impulse control describes a lack of control or ability to regulate actions related to inner urges. Impaired judgment and insight may include a lack of recognition of current or past symptoms, denial of need for treatment, or lack of recognition of consequences of actions. Levels of severity may be described as ⁽¹⁾⁽²⁾⁽³⁾:
 - Minimal (subtle symptoms that may rarely bother the patient and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult or **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult or **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult or **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are present on an almost constant basis, and contribute to **Serious** dysfunction in daily living for adult or **Serious** dysfunction in daily living for child or adolescent but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

1. Koita J, Riggio S, Jagoda A. The mental status examination in emergency practice. Emergency Medicine Clinics of North America 2010;28(3):439-51. DOI: 10.1016/j.emc.2010.03.008.
2. Baker SN. Management of acute agitation in the emergency department. Advanced Emergency Nursing Journal 2012 Oct-Dec;34(4):306-18; quiz 319-20. DOI: 10.1097/TME.0b013e31826f12d6.
3. Disruptive, impulse-control, and conduct disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:521-541.

Mania (adult)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hypervolubal or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (either socially, at work, or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences (eg, engaging in unrestrained buying sprees, sexual indiscretions, or foolish business investments). Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition). Symptom severity may be described as ⁽¹⁾⁽²⁾⁽³⁾⁽⁴⁾⁽⁵⁾⁽⁶⁾:
 - Mild (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are occasionally present)
 - Mild to moderate (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are present up to half the days of each week)
 - Moderate (periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms that are present more than half the days of each week)
 - Moderately severe (frequent, but not daily, periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)
 - Severe (daily periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)

References

1. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.

2. Altman EG, Hedeker D, Peterson JL, Davis JM. The Altman self-rating mania scale. *Biological Psychiatry* 1997;42(10):948-55. DOI: 10.1016/S0006-3223(96)00548-3.
3. Young RC, Biggs JT, Ziegler VE, Meyer DA. A rating scale for mania: reliability, validity and sensitivity. *British Journal of Psychiatry* 1978;133:429-35.
4. Altman Self-Rating Mania Scale. [Internet] Psychology Tools. Accessed at: <https://psychology-tools.com/test/altman-self-rating-mania-scale>. Updated 2023 [accessed 2023 Aug 30]
5. Young Mania Rating Scale. [Internet] Psychology Tools. Accessed at: <https://psychology-tools.com/test/young-mania-rating-scale>. Updated 2023 [accessed 2023 Sep 21]
6. Bipolar and related disorders. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed.* American Psychiatric Association; 2022:139-175.

Mania (child or adolescent)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hyperverbal or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (eg, socially or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences. Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition).(1)(2)

References

1. Pavuluri MN, Henry DB, Devineni B, Carbray JA, Birmaher B. Child mania rating scale: development, reliability, and validity. *Journal of the American Academy of Child and Adolescent Psychiatry* 2006;45(5):550-60. DOI: 10.1097/01.chi.0000205700.40700.50.
2. West AE, Celio CI, Henry DB, Pavuluri MN. Child Mania Rating Scale-Parent Version: a valid measure of symptom change due to pharmacotherapy. *Journal of Affective Disorders* 2011;128(1-2):112-9. DOI: 10.1016/j.jad.2010.06.013.

Mild dysfunction in daily living for adult

- **Mild** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some deterioration in interpersonal interactions (eg, increased conflict), but able to maintain some meaningful relationships
 - Some recent minor disruptions in self-care
 - Minor but consistent disruptions in social role functioning and meeting obligations, but still able to maintain these roles
 - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
 - Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed.* American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. *International Clinical Psychopharmacology* 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild dysfunction in daily living for child or adolescent

- **Mild** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Minor deterioration or episodic failure in relationships with peers, family, or adults (eg, defiance, lying, provocative behavior)
 - Self-care sporadically below usual or expected standards
 - Impairments in function characterized by ability to show improvements following episodes of deterioration
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
 - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
 - Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). *Archives of General Psychiatry* 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. *Psychiatry Research* 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Mild to moderate dysfunction in daily living for adult

- **Mild to moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some recent deterioration in interpersonal interactions (eg, conflicted, withdrawn), but maintains control of impulsive or abusive behaviors and maintains some meaningful relationships
 - Some recent disruptions in self-care below usual or expected standards

- Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health
- Some deterioration in social role functioning and meeting obligations (eg, avoiding responsibilities on some occasions), but still can maintain roles overall
- Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild to moderate dysfunction in daily living for child or adolescent

- **Mild to moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Some difficulty in relationships with peers, family, or adults (eg, defiance, lying), but without physical aggression
 - Self-care sometimes below usual or expected standards
 - Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health
 - Deteriorating school behavior such that disruption in school is threatened (eg, threat of expulsion)
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
 - Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Moderate dysfunction in daily living for adult

- **Moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Recent conflicted, withdrawn, or troubled relationships, but maintains control of impulsive, aggressive, or abusive behaviors
 - Self-care frequently below usual or expected standards
 - Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
 - Significant deterioration in ability to fulfill responsibilities (eg, work, commitments to significant others), including neglect or avoidance of these responsibilities on some occasions
 - Ongoing and variably severe deficits in interpersonal relationships and ability to engage socially in constructive manner
 - Other evidence of moderate dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Moderate dysfunction in daily living for child or adolescent

- **Moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Troubled relationships with peers, family, or adults (eg, conflicted, withdrawn), but without physical aggression
 - Self-care frequently below usual or expected standards
 - Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
 - Deteriorated school behavior such that disruption in school has occurred (eg, suspension, expulsion), and child is at risk for placement in alternative school setting or repeating grade
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of moderate dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Negative symptoms (adult)

• Negative symptoms represent thoughts, feelings, or behaviors that normally would be present, but are absent or diminished in relation to a psychiatric disorder. They may include apathy, anhedonia, poverty of speech, motor behavior that is diminished or signs or symptoms of psychomotor retardation.(1)(2)(3) Levels of severity may be described as:

- Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
- Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult)
- Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult)
- Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult)
- Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
- Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
- Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

References

1. Kay SR, Fiszbein A, Opler LA. The positive and negative syndrome scale (PANSS) for schizophrenia. Schizophrenia Bulletin 1987;13(2):261-276.
2. The PANSS Institute. [Internet] The PANSS Institute. Accessed at: <http://www.panss.org/>. Updated 2023 [accessed 2023 Sep 12]
3. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.

Negative symptoms (child or adolescent)

• Negative symptoms represent thoughts, feelings, or behaviors that normally would be present, but are absent or diminished in relation to a psychiatric disorder. They may include apathy, anhedonia, poverty of speech, motor behavior that is diminished or signs or symptoms of psychomotor retardation. Instruments are available for assessing severity of symptoms in young people, including the instrument for positive and negative symptoms in severely disturbed children and adolescents (Kiddie-PANSS).(1)(2) Levels of severity may be described as:

- Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
- Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for child or adolescent)
- Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
- Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for child or adolescent)
- Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
- Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
- Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
2. Fields JH, et al. Assessing positive and negative symptoms in children and adolescents. American Journal of Psychiatry 1994;151(2):249-53.

Obsessions or compulsions (adult)

• Obsessions are recurrent and persistent thoughts, urges, or images that are intrusive and unwanted, which often cause marked anxiety or distress, and are associated with attempts to ignore, suppress, or neutralize these experiences.(1)(2) Compulsions are repetitive behaviors that the individual feels driven to perform in response to an obsession or according to rules that must be applied rigidly; the behaviors or mental acts are aimed at preventing or reducing anxiety or distress, or preventing some dreaded event or situation, but are not connected in a realistic way with what they are designed to neutralize or prevent (or are clearly excessive).(1)(2)

References

1. Obsessive-compulsive and related disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:263-294.
2. Goodman WK, et al. The Yale-Brown Obsessive Compulsive Scale. I. Development, use, and reliability. Archives of General Psychiatry 1989;46(11):1006-11.

Obsessions or compulsions (child or adolescent)

- Obsessions are recurrent and persistent thoughts, urges, or images that are intrusive and unwanted, which often cause marked anxiety or distress, and are associated with attempts to ignore, suppress, or neutralize these experiences.(1)(2) Compulsions are repetitive behaviors that the individual feels driven to perform in response to an obsession or according to rules that must be applied rigidly; the behaviors or mental acts are aimed at preventing or reducing anxiety or distress, or preventing some dreaded event or situation, but are not connected in a realistic way with what they are designed to neutralize or prevent (or are clearly excessive). Young children may not be able to articulate the aims of these behaviors or mental acts.(1)(2)(3)

References

1. Obsessive-compulsive and related disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:263-294.
2. Goodman WK, et al. The Yale-Brown Obsessive Compulsive Scale. I. Development, use, and reliability. Archives of General Psychiatry 1989;46(11):1006-11.
3. Lopez-Pina JA, et al. Reliability generalization study of the Yale-Brown Obsessive-Compulsive Scale for children and adolescents. Journal of Personality Assessment 2015;97(1):42-54. DOI: 10.1080/00223891.2014.930470.

Orthostatic hypotension

- Orthostatic hypotension,^{[A][B]} as indicated by **1 or more** of the following(1)(2)(3):
 - Fall in SBP of 20 mm Hg or more 1 to 3 minutes after patient sits or stands from recumbent position
 - Fall in DBP of 10 mm Hg or more 1 to 3 minutes after patient sits or stands from recumbent position

References

1. Shibao C, Lipsitz LA, Biaggioni I, American Society of Hypertension Writing Group. Evaluation and treatment of orthostatic hypotension. Journal of the American Society of Hypertension 2013 Jul-Aug;7(4):317-324. DOI: 10.1016/j.jash.2013.04.006.
2. Dalal AS, Van Hare GF. Syncope. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:566-571.e1.
3. Fang JC, O'Gara PT. History and physical examination: an evidence-based approach. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine. 12th ed. Elsevier; 2022:123-140.

Footnotes

- A. Concomitant measurements of the heart rate are important to measure to help diagnose subtypes of orthostatic hypotension (eg, the lack of a compensatory increase in heart rate is typical of autonomic failure and an exaggerated tachycardia may be reflective of volume depletion). However, the heart rate is not a component of the definition of orthostatic hypotension which relies upon blood pressure alone.(1)(2)(3)
- B. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.

Other emotional or behavioral disturbance

- Emotional or behavioral disturbances impacting symptoms or function, including **1 or more** of the following^{[A](3)(4)(5)}:
 - Externalizing symptoms (eg, angry outbursts, physical or verbal aggression, oppositional defiant traits, agitation, anxiety, or other disruptive behaviors)
 - Internalizing symptoms (eg, sulking, rumination, anhedonia, dysphoria, apathy)
 - Evidence of severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - High levels of family conflict or interpersonal conflict
 - Escalating relapse behaviors
 - Other emotional or behavioral disturbance relevant to clinical presentation

References

1. Kuhn TM, Ebert JS, Gracey KA, Chapman GL, Epstein RA. Evidence-based interventions for adolescents with disruptive behaviors in school-based settings. Child and Adolescent Psychiatric Clinics of North America 2015;24(2):305-17. DOI: 10.1016/j.chc.2014.11.005.
2. Kennedy A, Cloutier P, Glennie JE, Gray C. Establishing best practice in pediatric emergency mental health: a prospective study examining clinical characteristics. Pediatric Emergency Care 2009;25(6):380-6. DOI: 10.1097/PEC.0b013e3181a79223.
3. Chun TH, Katz ER, Duffy SJ, Gerson RS. Challenges of managing pediatric mental health crises in the emergency department. Child and Adolescent Psychiatric Clinics of North America 2015;24(1):21-40. DOI: 10.1016/j.chc.2014.09.003.
4. Kober D, Martin A. Inpatient psychiatric, partial hospital, and residential treatment for children and adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3797-3803.
5. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306.

Footnotes

- A. Symptoms may be circumscribed (eg, attention-seeking behaviors) or may be seen in the context of one or more major psychiatric disorders, including depression; anxiety disorders; obsessive-compulsive disorder; posttraumatic stress disorder; eating disorders; attention-deficit hyperactivity disorders; bipolar disorders; schizophrenia; personality disorders; autism spectrum disorder and other disruptive, impulse-control, and conduct disorders.(1)(2)(3)

Positive symptoms (adult)

- Positive symptoms are feelings, thoughts, perceptions, or behaviors that usually are not present. They may include hallucinations, delusions, or other symptoms that are abnormal or bizarre. Levels of severity may be described as(1)(2)(3)(4):

- Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
- Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for adult)
- Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for adult)
- Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for adult)
- Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
- Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
- Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

References

1. Kay SR, Fiszbein A, Opler LA. The positive and negative syndrome scale (PANSS) for schizophrenia. *Schizophrenia Bulletin* 1987;13(2):261-276.
2. The PANSS Institute. [Internet] The PANSS Institute. Accessed at: <http://www.panss.org/>. Updated 2023 [accessed 2023 Sep 12]
3. Assessment measures. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders*. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
4. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders*. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.

Positive symptoms (child or adolescent)

- Positive symptoms are feelings, thoughts, perceptions, or behaviors that usually are not present. They may include hallucinations, delusions, or other symptoms that are abnormal or bizarre. Instruments are available for assessing severity of symptoms in young people, including the instrument for positive and negative symptoms in children and adolescents (Kiddie-PANSS).(1)(2) Levels of severity may be described as:
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. *Diagnostic and Statistical Manual of Mental Disorders*. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
2. Fields JH, et al. Assessing positive and negative symptoms in children and adolescents. *American Journal of Psychiatry* 1994;151(2):249-53.

Psychiatric symptoms (adult)

- Psychiatric symptoms, including **1 or more** of the following(1):
 - Hallucinations (adult), Delusions (adult), or other Positive symptoms (adult)
 - Negative symptoms (adult), including social or emotional withdrawal or lack of self-initiated behavior or movement
 - Mania (adult)
 - Depressive symptoms (adult)
 - Anxiety (adult)
 - Hyperactivity or inattention (adult)
 - Obsessions or compulsions (adult)
 - Personality disorders/maladaptive personality traits
 - Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder

References

1. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.

Psychiatric symptoms (child or adolescent)

- Psychiatric symptoms, including **1 or more** of the following(1):
 - Hallucinations (child or adolescent), Delusions (child or adolescent), or other Positive symptoms (child or adolescent)
 - Negative symptoms (child or adolescent), including social or emotional withdrawal or lack of self-initiated behavior or movement
 - Mania (child or adolescent)
 - Depressive symptoms (child or adolescent)
 - Anxiety (child or adolescent)
 - Hyperactivity or inattention (child or adolescent)

- Obsessions or compulsions (child or adolescent)
- Personality disorders/maladaptive personality traits
- Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder

References

1. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.

Psychiatric, behavioral, or other comorbid conditions for adult

- Psychiatric, behavioral, or other comorbid conditions for adult are appropriate for management at proposed level of care, as indicated by **1 or more** of the following[A]:

- Psychiatric symptoms (adult)
- Comorbid biomedical or developmental condition[B][C]
- Comorbid substance use disorder[D][E]
- Cognitive or memory impairment[F]
- Impaired impulse control, judgment, or insight
- Other emotional or behavioral disturbance

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
3. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
4. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
5. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
6. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.
7. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306.
8. Service planning and placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:105-126.
9. Douzenis A, et al. Factors affecting hospital stay in psychiatric patients: the role of active comorbidity. BMC Health Services Research 2012;12:166. DOI: 10.1186/1472-6963-12-166.
10. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.
11. Abadi M, et al. Achieving whole health: a preliminary study of TCMLH, a group-based program promoting self-care and empowerment among Veterans. Health Education & Behavior 2022;49(2):347-357. DOI: 10.1177/10901981211011043.
12. Joshi H, Corcoran-Lozano P. The behavioral health consultant: roles and responsibilities. Pediatric Clinics of North America 2021;68(3):563-571. DOI: 10.1016/j.pcl.2021.02.002.
13. Rosenbaum E, Gordon AE, Cresta J, Shaughnessy AF, Jonas WB. Implementing whole person primary care. Annals of Family Medicine 2023;21(2):Online. DOI: 10.1370/afm.2952.
14. Pourat N, et al. Final Evaluation of California's Whole Person Care (WPC) Program. [Internet] UCLA Center for Health Policy Research. 2022 Dec Accessed at: <https://healthpolicy.ucla.edu/publications/Pages/default.aspx>. [accessed 2023 Sep 08]
15. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104.
16. Intake and assessment. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:39-68.
17. Substance Abuse: Clinical Issues in Intensive Outpatient Treatment. Treatment Improvement Protocol (TIP) Series, No. 47 [Internet] Center for Substance Abuse Treatment. 2006 Accessed at: <https://www.samhsa.gov/>. [accessed 2023 Aug 11]
18. Seitz DP, et al. Characteristics of older adults hospitalized in acute psychiatric units in Ontario: a population-based study. Canadian Journal of Psychiatry 2012;57(9):554-63.
19. Nasreddine ZS, et al. The Montreal Cognitive Assessment, MoCA: a brief screening tool for mild cognitive impairment. Journal of the American Geriatrics Society 2005;53(4):695-9. DOI: 10.1111/j.1532-5415.2005.53221.x.
20. Gierus J, Mosiolek A, Kowesko T, Wnukiewicz P, Kozyna O, Szulc A. The Montreal Cognitive Assessment as a preliminary assessment tool in general psychiatry: Validity of MoCA in psychiatric patients. General Hospital Psychiatry 2015;37(5):476-80. DOI: 10.1016/j.genhosppsych.2015.05.011.

Footnotes

- A. Determination of severity may be based on symptom severity alone (including intensity or frequency of symptoms and the extent to which they interfere with functioning), or may be due to other factors (eg, comorbid medical illness, developmental condition, cognitive impairment, substance use disorder, or other factors that contribute to destabilization or decreased ability to cope with the underlying behavioral health disorder). If the comorbidity affects the level of care appropriate to meet the patient's behavioral health needs, the manner in which it impacts the behavioral health condition (and how the comorbidity will be managed/treated at the proposed level of care) should be documented in order to optimize comprehensive patient care. Assessment for comorbid conditions should take place at admission evaluation, and any identified conditions should be manageable/treatable by the program (either directly or through alternative arrangements). Co-occurring mental health and substance use conditions and co-occurring medical and behavioral health conditions are considered an expectation, not an exception, in all programs at all psychiatric and addiction service levels of care. Integrated care, with recovery-oriented and "co-occurring capable" services, is increasingly expected to be included in the design of programs at all levels of care and in all settings. In programs without integrated care, comorbidities that require more active management/treatment or that currently distract the patient from treatment or have been associated with challenges in treatment follow-through in the past should prompt consideration of a program designed to provide treatment for both disorders (eg, a dual-diagnosis program).

- B. A significant comorbid biomedical or developmental condition has the potential to prolong the course of illness, or necessitate admission to a higher level of care or closer monitoring of patients requiring psychiatric care.(1)(3)(4)(5)(9)
- C. Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(4)(5)(10) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(11)(12)(13)(14) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(1)(3)(4)(5)(15)
- D. A significant comorbid substance use disorder (either reflecting comorbidity in a patient whose primary diagnosis is a psychiatric disorder, such as an affective disorder, or is present in a patient with a polysubstance use disorder) has the potential to impact the appropriate level of care in which the underlying condition is managed, or require consideration of a program with structures in place to treat multiple conditions, such as a dual-diagnosis program. The risk for problems with substance use is increased with the presence of many types of behavioral disturbance (eg, impulse control disorders, or self-medication of depression or anxiety symptoms). The severity of the substance use disorder as it relates to the underlying psychiatric condition includes the frequency and chronicity of use, ability to control use, intensity of other symptoms (eg, cravings, withdrawal), or other negative impact on the underlying condition.(1)(3)(4)(5)(7)(8)
- E. Evaluation/assessment and treatment of co-occurring substance use disorders through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(4)(5) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(11)(12)(13)(14) The level that comorbid substance use disorders contribute to the primary presenting condition may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care or specialty dual-diagnosis program).(1)(3)(4)(5)(15)
- F. Intake and assessment should include a mental status examination and memory assessment. If cognitive impairments are identified, they may require modifying the intensity or pace of planned interventions to allow the patient to participate in the program, or using an enhanced program designed to address co-occurring cognitive disorders.(8)(16)(17)(18)(19)(20)

Psychiatric, behavioral, or other comorbid conditions for child or adolescent

- Psychiatric, behavioral, or other comorbid conditions for child or adolescent are appropriate for management at proposed level of care, as indicated by **1 or more** of the following^[A]:
 - Psychiatric symptoms (child or adolescent)
 - Comorbid biomedical or developmental condition^[B]^[C]
 - Comorbid substance use disorder^[D]^[E]
 - Cognitive or memory impairment^[F]
 - Impaired impulse control, judgment, or insight
 - Other emotional or behavioral disturbance

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet]. American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet]. American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
3. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet]. American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
4. CASII. Child and Adolescent Service Intensity Instrument [Internet]. American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
5. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
6. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.
7. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306.
8. Service planning and placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:105-126.
9. Douzenis A, et al. Factors affecting hospital stay in psychiatric patients: the role of active comorbidity. BMC Health Services Research 2012;12:166. DOI: 10.1186/1472-6963-12-166.
10. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.
11. Abadi M, et al. Achieving whole health: a preliminary study of TCMLH, a group-based program promoting self-care and empowerment among Veterans. Health Education & Behavior 2022;49(2):347-357. DOI: 10.1177/10901981211011043.
12. Joshi H, Corcoran-Lozano P. The behavioral health consultant: roles and responsibilities. Pediatric Clinics of North America 2021;68(3):563-571. DOI: 10.1016/j.pcl.2021.02.002.
13. Rosenbaum E, Gordon AE, Cresta J, Shaughnessy AF, Jonas WB. Implementing whole person primary care. Annals of Family Medicine 2023;21(2):Online. DOI: 10.1370/afm.2952.
14. Pourat N, et al. Final Evaluation of California's Whole Person Care (WPC) Program. [Internet]. UCLA Center for Health Policy Research. 2022 Dec Accessed at: <https://healthpolicy.ucla.edu/publications/Pages/default.aspx>. [accessed 2023 Sep 08]
15. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104.
16. Intake and assessment. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:39-68.
17. Substance Abuse: Clinical Issues in Intensive Outpatient Treatment. Treatment Improvement Protocol (TIP) Series, No. 47 [Internet]. Center for Substance Abuse Treatment. 2006 Accessed at: <https://www.samhsa.gov/>. [accessed 2023 Aug 11]
18. Seitz DP, et al. Characteristics of older adults hospitalized in acute psychiatric units in Ontario: a population-based study. Canadian Journal of Psychiatry 2012;57(9):554-63.
19. Nasreddine ZS, et al. The Montreal Cognitive Assessment, MoCA: a brief screening tool for mild cognitive impairment. Journal of the American Geriatrics Society 2005;53(4):695-9. DOI: 10.1111/j.1532-5415.2005.53221.x.
20. Gierus J, Mosiolek A, Kowesko T, Wnukiewicz P, Kozyra O, Szulc A. The Montreal Cognitive Assessment as a preliminary assessment tool in general psychiatry: Validity of MoCA in psychiatric patients. General Hospital Psychiatry 2015;37(5):476-80. DOI: 10.1016/j.genhosppsych.2015.05.011.

Footnotes

- A. Determination of severity may be based on symptom severity alone (including intensity or frequency of symptoms and the extent to which they interfere with functioning), or may be due to other factors (eg, comorbid medical illness, developmental condition, cognitive impairment, substance use disorder, or other factors that contribute to destabilization or decreased ability to cope with the underlying behavioral health disorder). If the comorbidity affects the level of care appropriate to meet the patient's behavioral health needs, the manner in which it impacts the behavioral health condition (and how the comorbidity will be managed/treated at the proposed level of care) should be documented in order to optimize comprehensive patient care. Assessment for comorbid conditions should take place at admission evaluation, and any identified conditions should be manageable/treatable by the program (either directly or through alternative arrangements). Co-occurring mental health and substance use conditions and co-occurring medical and behavioral health conditions are considered an expectation, not an exception, in all programs at all psychiatric and addiction service levels of care. Integrated care, with recovery-oriented and "co-occurring capable" services, is increasingly expected to be included in the design of programs at all levels of care and in all settings. In programs without integrated care, comorbidities that require more active management/treatment or that currently distract the patient from treatment or have been associated with challenges in treatment follow-through in the past should prompt consideration of a program designed to provide treatment for both disorders (eg, a dual-diagnosis program).(1)(2)(3)(4)(5)(6)(7)(8)
- B. A significant comorbid biomedical or developmental condition has the potential to prolong the course of illness, or necessitate admission to a higher level of care or closer monitoring of patients requiring psychiatric care.(1)(3)(4)(5)(9)
- C. Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(4)(5)(10) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(11)(12)(13)(14) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(1)(3)(4)(5)(15)
- D. A significant comorbid substance use disorder (either reflecting comorbidity in a patient whose primary diagnosis is a psychiatric disorder, such as an affective disorder, or is present in a patient with a polysubstance use disorder) has the potential to impact the appropriate level of care in which the underlying condition is managed, or require consideration of a program with structures in place to treat multiple conditions, such as a dual-diagnosis program. The risk for problems with substance use is increased with the presence of many types of behavioral disturbance (eg, impulse control disorders, or self-medication of depression or anxiety symptoms). The severity of the substance use disorder as it relates to the underlying psychiatric condition includes the frequency and chronicity of use, ability to control use, intensity of other symptoms (eg, cravings, withdrawal), or other negative impact on the underlying condition.(1)(3)(4)(5)(7)(8)
- E. Evaluation/assessment and treatment of co-occurring substance use disorders through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(4)(5) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(11)(12)(13)(14) The level that comorbid substance use disorders contribute to the primary presenting condition may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care or specialty dual-diagnosis program).(1)(3)(4)(5)(15)
- F. Intake and assessment should include a mental status examination and memory assessment. If cognitive impairments are identified, they may require modifying the intensity or pace of planned interventions to allow the patient to participate in the program, or using an enhanced program designed to address co-occurring cognitive disorders.(8)(16)(17)(18)(19)(20)

Respiratory distress

- Respiratory distress, as indicated by **ALL** of the following(1)(2):
 - Patient with **1 or more** of the following:
 - Dyspnea (difficulty breathing)
 - Tachypnea
 - Abnormal breathing pattern (eg, chest retractions)
 - Other evidence of difficulty breathing
 - Evidence of respiratory compromise, as indicated by **1 or more** of the following:
 - Hypoxemia
 - Altered mental status
 - Other evidence of respiratory compromise (eg, respiratory acidosis)

References

1. Braithwaite SA, Wessel AL. Dyspnea. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:194-201.e2.
2. Naureckas ET, Solway J. Disturbances of respiratory function. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:2133-2139.

Serious dysfunction in daily living for adult

- **Serious** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Serious deterioration in quality of interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care (eg, personal hygiene, appearance) to usual standards
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform close to usual standards in adult obligations (eg, work, parenting) and neglecting these responsibilities frequently or over extended period of time
 - Other evidence of serious dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Serious dysfunction in daily living for child or adolescent

- **Serious** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Serious deterioration in interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care as appropriate to age or developmental level
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform adequately in school (including specialized setting) due to disruptive or aggressive behavior
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of serious dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Severe dysfunction in daily living for adult

- **Severe** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in social interactions (eg, threatening behaviors with little or no provocation)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Complete inability to maintain any appropriate aspect of personal responsibility in any adult roles (eg, occupational, parental)
 - Other evidence of severe dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Severe dysfunction in daily living for child or adolescent

- **Severe** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in interactions with peers, adults, or family (eg, assaultive behaviors with no provocation, high levels of family conflict)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Nearly complete inability to maintain any appropriate school behavior or academic achievement
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of severe dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.

7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Sufficient relief

- Sufficient relief is that which is adequate to alleviate the immediate impetus for recurrence of suicide attempt or act of harm to self or another.(1)(2)(3)

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. Petti TA. Milieu treatment: inpatient, partial hospitalization, and residential programs. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:1019-1042.
3. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Suicide attempt

- A suicide attempt may be identified by an actual, interrupted, or aborted act of self-injurious behavior accompanied by **1 or more** of the following(1)(2)(3):
 - Explicit or implicit evidence that the person believed the act would be lethal (eg, gaining access to gun, carbon monoxide inhalation)
 - Explicit or implicit evidence that the person intended to die (eg, act that was unlikely to be discovered in time for rescue, writing suicide note)

References

1. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
2. Sewall CJR, Goldstein TR, Brent DA. Youth suicide. In: Dulcan MK, editor. Dulcan's Textbook of Child and Adolescent Psychiatry. 3rd ed. American Psychiatric Association Publishing; 2022:551-564.
3. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.

Tachycardia

- Tachycardia,[A][B] as indicated by **1 or more** of the following(1)(2):
 - Heart rate greater than 100 beats per minute in adult or child age 6 years or older
 - Heart rate greater than 115 beats per minute in child 3 to 5 years of age
 - Heart rate greater than 125 beats per minute in child 1 or 2 years of age
 - Heart rate greater than 130 beats per minute in infant 6 to 11 months of age
 - Heart rate greater than 150 beats per minute in infant 3 to 5 months of age
 - Heart rate greater than 160 beats per minute in infant 1 or 2 months of age

References

1. Southmayd GL. Tachycardia. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:729-739.
2. Pediatric parameters and equipment. In: Kleinman K, McDaniel L, Molloy M, editors. The Harriet Lane Handbook: A Manual for Pediatric House Officers. 22nd ed. 202: Elsevier; 2021:frontpiece tables.

Footnotes

- A. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. Interpretation of heart rate requires clinical judgment and consideration of several patient-specific factors, such as the patient's baseline heart rate, medications, and clinical impact. For example, an elderly patient on a beta-blocker medication with a baseline resting heart rate of 60 beats per minute may be clinically tachycardic at a heart rate of 94 beats per minute. Likewise, a patient who is upset, in pain, or nervous in the emergency department with a heart rate of 106 beats per minute may meet the technical definition of tachycardia, but this tachycardia (absent associated findings such as chest pain or hypotension) may not be clinically important. The numeric values included in this definition are provided to allow for consistency in terms of a technical definition of the term tachycardia. Whether a heart rate above or below the technical threshold is clinically meaningful is a matter of persistence, context, and clinical judgment.

Tachypnea

- Tachypnea,[A][B] as indicated by respiratory rate of **1 or more** of the following(2)(3):
 - Greater than 18 breaths per minute in adult or child 13 years of age or older[A]
 - Greater than 22 breaths per minute in child 6 to 12 years of age[A]
 - Greater than 25 breaths per minute in child 3 to 5 years of age[A]
 - Greater than 30 breaths per minute in child 1 or 2 years of age[A]
 - Greater than 40 breaths per minute in infant 6 to 11 months of age[A]
 - Greater than 45 breaths per minute in infant 3 to 5 months of age[A]
 - Greater than 60 breaths per minute in infant 1 or 2 months of age[A]

References

1. Schrager DL. Approach to the patient with abnormal vital signs. In: Goldman L, Schafer AI, editors. Goldman-Cecil Medicine. 26th ed. Elsevier; 2020:28-31.e2.
2. Braithwaite SA, Wessel AL. Dyspnea. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:194-201.e2.
3. Pediatric parameters and equipment. In: Kleinman K, McDaniel L, Molloy M, editors. The Harriet Lane Handbook: A Manual for Pediatric House Officers. 22nd ed. 202: Elsevier; 2021:frontpiece tables.

Footnotes

- A. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. Interpretation of respiratory rate requires clinical judgment and consideration of several patient-specific factors, such as the patient's baseline respiratory rate and whether the respiratory rate is indicative of significant underlying respiratory or other pathology. A mildly increased respiratory rate (eg, 20 to 22 in an adult), absent other indications of serious illness (eg, hypoxemia, metabolic acidosis, decreased tidal volume, pain), may be transitory and not clinically important. The numeric values included in this definition are provided to allow for consistency in terms of a technical definition of the term tachypnea. Whether a respiratory rate above the technical threshold is clinically meaningful is a matter of persistence, context, and clinical judgment. In addition, the measurement of the respiratory rate is subject to error. A medical textbook states that because there can be significant breath to breath variation in respiratory rate, the rate should be assessed for at least 30 seconds, preferably for a full minute.⁽¹⁾ This same textbook also states that new technologies designed to measure the respiratory rate have not proved to be clinically useful.⁽¹⁾

Thoughts of suicide

- Thoughts of suicide or suicidal ideations are defined as "thoughts of serving as the agent of one's own death," to be distinguished from thoughts of death that do not involve actively bringing death about.⁽¹⁾⁽²⁾

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

MCG Health
Behavioral Health Care 28th Edition
Copyright © 2024 MCG Health, LLC
All Rights Reserved

Last Update: 3/14/2024 2:11:55 AM
Build Number: 28.1.20240313235330.016020